

Started on	Sunday, 12 February 2023, 9:01 PM
State	Finished
Completed on	Sunday, 12 February 2023, 9:01 PM
Time taken	18 secs
Marks	0.00/110.00
Grade	0.00 out of 100.00



Question 1

Not answered

Marked out of 1.00

Many recreational drugs are associated with a degree of dependence, either physiological or psychological. Which of the following substances has the lowest risk of dependence?

Select one:

- ☐ Mephedrone
- ☐ Cocaine
- ☐ MDMA
- ☐ LSD
- ☐ Cannabis

Your answer is incorrect.

LSD has the lowest risk of dependence of the drugs listed. "Abuse of LSD is rather difficult; the drug produces such an absurd high that daily ingestion is almost impossible. Thus LSD use does not lead to physical dependence. However, the tolerance as mentioned disappears after a few days of abstinence without producing craving. So, LSD dependence is typically psychological and not physical [Gable, 1993; Bogenschutz et al. 2015]". The other substances mentioned have, to a greater or lesser extent, a significant risk of psychological dependence. Ref: Excerpt from Das S, et al. Lysergic acid diethylamide: a drug of 'use?'. Therapeutic advances in psychopharmacology. 2016 Jun;6(3):214-28.

The correct answer is: LSD

Question 2

Not answered

Marked out of 1.00

Staccato speech is most commonly associated with;

Select one:

- ☐ Frontal lobe disease
- ☐ Cerebellar disease
- ☐ Parietal lobe disease
- ☐ 9th cranial nerve palsy
- ☐ Temporal lobe disease

Your answer is incorrect.

Staccato speech is when an individual speaks in fragments of sentences that are punctuated by pauses, which interrupt sentence, to the point of destroying the flow of speech. Such speech is abrupt, broken, and usually quite hard to follow. It is associated with cerebellar disease. Ref: Lechtenberg R, Gilman S. Speech disorders in cerebellar disease. Annals of Neurology. 1978 Apr;3(4):285-90.

The correct answer is: Cerebellar disease



Question 3

Not answered

Marked out of 1.00

Motivational interviewing is commonly used in substance misuse services and other health settings to assist patients in making positive changes. Which of the following does it involve?

Select one:

- ☐ Understanding unhelpful cognitions
- ☐ Reflecting on past experiences
- ☐ Developing discrepancy
- ☐ Acceptance of self
- ☐ Homework tasks

Your answer is incorrect.

Motivational interviewing is a directive, patient-centred counselling method which aims to enhance an individual's desire to make positive changes. It involves: expressing empathy, supporting and developing discrepancy, dealing with resistance, supporting self-efficacy and developing autonomy. Ref; <https://www.ncbi.nlm.nih.gov/books/NBK64964/>

The correct answer is: Developing discrepancy



Question 4

Not answered

Marked out of 1.00

Mr Harry Clyde is a 45-year-old man with a long history of alcohol misuse. His family have brought him to his local A&E as they were concerned about him. He has subsequently been diagnosed with Korsakoff Syndrome. Which of the following is the most common symptom of this disorder?

Select one:

- ☐ Receptive dysphasia
- ☐ Amnesia
- ☐ Attentional impairment
- ☐ Confabulation
- ☐ Expressive dysphasia

Your answer is incorrect.

Anterograde amnesia and a variable degree of retrograde amnesia are the most common symptoms of Korsakoff's syndrome. Confabulation can occur as a consequence but is not seen in all cases. Ref: Lishman's Organic Psychiatry

The correct answer is: Amnesia



Question 5

Not answered

Marked out of 1.00

Sarah Hughes is a 25-year-old woman with a 2-hour-old newborn who is noted to be very irritable, restless, tremulous and inconsolably crying. Which of the following substances did her mother most likely use during pregnancy?

Select one:

- ☐ Cannabis
- ☐ Cocaine
- ☐ Heroin
- ☐ Alcohol
- ☐ Nicotine

Your answer is incorrect.

Heroin withdrawal tends to be seen within 24 hours of birth, as with other short acting opioids. Signs and symptoms include excessive crying, tremors, increased muscle tone and reflexes. Similar withdrawal presentations are seen with other drugs, although this is less frequent with alcohol and cannabis.

The correct answer is: Heroin



Question 6

Not answered

Marked out of 1.00

Buprenorphine is seen as a useful medication for opioid substitution as it has significantly different properties to methadone. In which of the following ways is buprenorphine different to methadone?

Select one:

- ☐ It has no risk of respiratory depression
- ☐ More marked withdrawals if abruptly stopped
- ☐ Buprenorphine has no ceiling effect
- ☐ Higher treatment retention
- ☐ Buprenorphine can be given orally or intravenously

Your answer is incorrect.

There are combined buprenorphine and naloxone preparations available for oral and intravenous administration although their efficacy is still unclear. Buprenorphine can cause respiratory depression in high doses or in associations with other psychotropics. It and methadone are generally given orally. Methadone has higher treatment retention and has more severe withdrawals if abruptly stopped. Buprenorphine has a ceiling effect with good safety profile. Methadone does not have ceiling effect and useful in patients dependent on high doses of opioids.

The correct answer is: Buprenorphine can be given orally or intravenously

Question 7

Not answered

Marked out of 1.00

Mr Howard Hughes is a 43-year-old man who has repeatedly attended his GP surgery and local A&E with concerns that something is blocking his throat. He has had numerous negative investigations, including direct visualisation of his pharynx, which can temporarily reassure him. Nevertheless, he continues to present himself. When asked, he describes a sense of something in his throat and is clearly very anxious. This is most likely to be

Select one:

- ☐ Somatic hallucination
- ☐ Hypochondriasis
- ☐ Somatisation disorder
- ☐ Delusional disorder
- ☐ Depression

Your answer is incorrect.

This is most likely to be hypochondriasis as the patient has an overvalued idea about a particular diagnosis. In somatisation disorder, patients describe numerous and often changeable physical symptoms. Patients experiencing psychotic symptoms would generally not be reassured, even temporarily, by negative investigation findings. Note that DSM-5 prefers the term Somatic Symptom Disorder for the same presentation.

The correct answer is: Hypochondriasis

Question 8

Not answered

Marked out of 1.00

Post-natal depression can lead to significant distress in new mothers and impair their ability to bond with and care for their newborn children. What percentage of new mothers suffers from postnatal depression?

Select one:

- ☐ 10-15%
- ☐ 15-20%
- ☐ 5-10%
- ☐ 0-5%
- ☐ 20-25%

Your answer is incorrect.

It is thought that 10-15% of new mothers experience post-natal depression, with rates reducing to <10% after 12 weeks postpartum. Ref: Leahy-Warren et al., Arch Psychiatr Nurs. 2011 Jun;25(3):174-84

The correct answer is: 10-15%



Question 9

Not answered

Marked out of 1.00

Inpatient suicide rates in the UK have reduced significantly since 2000. Which of the following is thought to have contributing most to this decrease?

Select one:

- ☐ Removal of potential ligature points
- ☐ Use of formal risk assessment tools
- ☐ Increased staffing levels
- ☐ Introduction of positive risk taking
- ☐ Introduction of enhanced care plans

Your answer is incorrect.

In the UK, hanging was the reported method in 77% of inpatient suicides between 1999 and 2007. The most common ligature points were doors, hooks or handles, windows, belts, and sheets or towels. Removal of ligature points had a significant impact on the rate of inpatient suicides, though the use of shoelaces, doors, and windows increased over time. Ref; Bowers, L.; Dack, C.; Gul, N.; Thomas, B.; James, K. (2011) Learning from prevented suicide in psychiatric inpatient care: An analysis of data from the National Patient Safety Agency. International Journal of Nursing Studies 48 (12) 1459-1465

The correct answer is: Removal of potential ligature points

Question 10

Not answered

Marked out of 1.00

People with schizophrenia can have certain cognitive impairments that are present when they are not experiencing an episode of psychosis. Which of the following is most common?

Select one:

- ☐ Impaired visual processing
- ☐ Impaired comprehension ability
- ☐ Impaired anterograde memory
- ☐ Impaired planning
- ☐ Ataxia

Your answer is incorrect.

The most common cognitive impairments in schizophrenia are those around executive functioning and working memory.

The correct answer is: Impaired planning



Question 11

Not answered

Marked out of 1.00

Mr Charlie Simms is a 26-year-old man who has recently been diagnosed with schizophrenia. Once he has recovered he is keen to know more about his prognosis and what factors might have an impact on it. Which of the following factors is most likely to support a favourable prognosis?

Select one:

- ☐ Slow insidious onset
- ☐ Family history of affective disorder
- ☐ Absence of affective symptoms
- ☐ Longer duration of untreated symptoms
- ☐ Younger age of onset

Your answer is incorrect.

A family history of affective disorder is associated with a positive prognosis, as is having affective symptoms during episodes of illness. Acute onset, short duration of untreated symptoms, older age of onset, good premorbid functioning, being in a stable relationship and having a good support system are all positive prognostic factors in schizophrenia.

The correct answer is: Family history of affective disorder

Question 12

Not answered

Marked out of 1.00

Puerperal psychosis is a psychotic illness that tends to occur shortly after a woman gives birth. Which of the following is the most common presentation?

Select one:

- ☐ Affective symptoms
- ☐ Auditory hallucinations
- ☐ Reduced activity
- ☐ Paranoid delusions
- ☐ Thought disorder

Your answer is incorrect.

Prominent affective symptoms and unusual behaviour are the most common presenting symptoms in puerperal psychosis. This is often associated with profound anxiety and this can be associated with persecutory ideation. Many patients later satisfy the criteria for psychotic bipolar disorder.

The correct answer is: Affective symptoms

Question 13

Not answered

Marked out of 1.00

Mr Edward Hughes is a 23-year-old man with autism spectrum disorder and moderate intellectual impairment. He lives in a placement and has been demonstrating aggressive behaviour over the last several months. Attempts to identify the cause of his distress or address his behaviour via non-pharmacological methods have been fruitless. Which of the following would be the most appropriate treatment for him?

Select one:

- ☐ Methylphenidate
- ☐ Atomoxetine
- ☐ Risperidone
- ☐ Fluoxetine
- ☐ Mirtazapine

Your answer is incorrect.

Risperidone and Aripiprazole have the greatest evidence base for the pharmacological management of aggression in ASD. Other second- and first-generation antipsychotics, antiepileptic medications, lithium, naltrexone and N-acetylcysteine have also been used although with less evidence being available for these agents.

The correct answer is: Risperidone



Question **14**

Not answered

Marked out of 1.00

Mr Neil Peterson is a 36-year-old man who is currently experiencing a moderate depressive episode. He is struggling to motivate himself to engage with his usual activities and is due to be assessed for behavioural activation therapy. Which of the following will this include?

Select one:

- ☐ Negative cognitions
- ☐ Impulsivity
- ☐ Logic errors
- ☐ Degree of avoidance
- ☐ Early life activities

Your answer is incorrect.

Assessment for behavioural activation therapy includes identification of TRAP (Trigger, Response, Avoidance Pattern) with the avoidance including direct avoidance, rumination and use of alcohol, drugs and other maladaptive coping strategies. The aim is to replace this with TRAC (Trigger, Response, Alternate Coping response). Ref: Weinstock LM, Munroe MK, Miller IW. Behavioral activation for the treatment of atypical depression: a pilot open trial. Behavior modification. 2011 Jul;35(4):403-24.

The correct answer is: Degree of avoidance



Question 15

Not answered

Marked out of 1.00

Mrs Susan Matthews is a 42-year-old woman with a previous diagnosis of bipolar affective disorder. She has been well without medication for three years but has now presented with a clear-cut episode of depression. Which of the following would be most appropriate to treat her?

Select one:

- ☐ Lithium
- ☐ Sertraline
- ☐ Amitriptyline
- ☐ Sodium valproate
- ☐ Quetiapine

Your answer is incorrect.

NICE recommends that individuals with a diagnosis of bipolar depression, who are not currently receiving medication, should be prescribed either Olanzapine and Fluoxetine in combo or Quetiapine by itself as first line treatment. Patients can choose to have Olanzapine without Fluoxetine if desired (but not the antidepressant alone). Lamotrigine can also be offered if the above therapies are ineffective or the patient expresses a preference for it. Ref: <https://www.nice.org.uk/guidance/cg185/>

The correct answer is: Quetiapine

Question **16**

Not answered

Marked out of 1.00

ECT is a highly effective intervention in those with severe episodes of depression, which are treatment resistant or associated with risk to life. Which of the following is true about ECT?

Select one:

- ☐ Seizures on EEG last longer than the physical seizure
- ☐ A history of Ischemic heart disease is an absolute contraindication for ECT
- ☐ ECT can only be delivered for inpatients
- ☐ An ECT-induced seizure lasting longer than 2 minutes should be pharmacologically aborted
- ☐ Antidepressants should be discontinued while ECT is delivered.

Your answer is incorrect.

It is common for seizure activity on the EEG to continue for longer than the physical movements, particularly as patients receiving ECT will have also have been administered a muscle relaxant. Ref: Fink M, Abrams R. EEG monitoring in ECT: a guide to treatment efficacy. *Psychiatry Times*, May. 1998 May 1;15(5).

The correct answer is: Seizures on EEG last longer than the physical seizure

Question 17

Not answered

Marked out of 1.00

Tina Keyes is a 2-year-old girl who was born with a cleft palate, which is in the process of being surgically repaired. She demonstrated evidence of development delay during her first year and it is now thought that she will likely have a degree of intellectual disability. Which of the following medications taken during pregnancy is most likely to have contributed to these problems?

Select one:

- ☐ Lithium
- ☐ Clozapine
- ☐ Sertraline
- ☐ Sodium valproate
- ☐ Diazepam

Your answer is incorrect.

Sodium valproate should not be prescribed to women of childbearing age unless other therapies are ineffective or inappropriate. If taken throughout pregnancy, about 10% of infants will have a congenital malformation and 40% will have a degree of developmental delay and intellectual impairment. Sertraline and other SSRIs are associated with a small increase in the risk of congenital malformations. Lithium was associated with an increase in the risk of congenital malformations, particularly Ebstein's anomaly. Benzodiazepines were considered to be associated with cleft lip and palate, but recent studies indicate that the risk is much lower than what was thought before. Antipsychotics, with the exception of risperidone, which shows a small increase in the risk of cardiac defects, are not associated with congenital abnormalities or developmental problems.

The correct answer is: Sodium valproate

Question **18**

Not answered

Marked out of 1.00

Mr Vincent Payes is a 38-year-old man with a diagnosis of paranoid schizophrenia. He has had a recent relapse, which has been successfully treated with Olanzapine, but he reports that it makes him rather sedated during the day, including when he is driving. It appears likely that it is impairing his ability to drive. Which of the following should be done?

Select one:

- ☐ Report the patient's inability to drive to the DVLA directly
- ☐ Advise the patient to stop driving and report to the DVLA themselves
- ☐ Stop the olanzapine
- ☐ Advise the patient to omit the Olanzapine if they have to drive the next day
- ☐ Advise the patient to continue to drive but be cautious

Your answer is incorrect.

The patient needs to stop driving if their medication may be impairing their ability to do so safely. It is usually appropriate to advise the patient to stop driving and inform the DVLA. It may then be appropriate to modify their prescription so they are not so sedated during the day, provided this does not compromise their mental wellbeing. If a patient will not inform the DVLA himself or herself, then it is appropriate to contact them directly. The patient's confidentiality can be breached for this purpose.

The correct answer is: Advise the patient to stop driving and report to the DVLA themselves

Question 19

Not answered

Marked out of 1.00

Mrs Yanica Presley is a 32-year-old woman with a previous diagnosis of bipolar affective disorder and has had several previous episodes of mania requiring admission. She has responded well to Lithium and has had no relapses since it was started 3 years ago. She is currently 36 weeks pregnant and has been taking Lithium 600mg throughout her pregnancy. She is currently well in her mood and is not planning on breastfeeding. Which of the following would be the most appropriate action?

Select one:

- ☐ Stop lithium immediately
- ☐ Do nothing and review after 1 week after delivery
- ☐ Increase dose to 800 mg
- ☐ Reduce dose to 400 mg, then repeat blood levels after 1 week
- ☐ Continue lithium and check levels weekly until delivery and for 2 weeks after

Your answer is incorrect.

Lithium can be continued throughout pregnancy provided that adequate monitoring is provided. This is usually 4 weekly until 34 weeks and then weekly until 2 weeks post-partum, with doses being modified as appropriate. An increase in dose is often required earlier in the pregnancy with this reversing to an extent in late pregnancy. Lithium can be omitted for 24-48 prior to labour to reduce effects on the newborn but this must be balanced against the risk of relapse in high risk women. Women at high risk of relapse in the post-partum must not be left without effective treatment.

The correct answer is: Continue lithium and check levels weekly until delivery and for 2 weeks after

Question 20

Not answered

Marked out of 1.00

Parkinsonism describes a collection of physical symptoms, which are usually caused by a lack of active dopamine in certain areas of the thalamus. It can occur as a consequence of antipsychotic medication. Which of the following is the Parkinsonian symptom most likely to be seen in association with antipsychotic use?

Select one:

- ☐ Unilateral symptoms
- ☐ Postural tremor
- ☐ Rigidity
- ☐ Pill rolling tremor
- ☐ Festination

Your answer is incorrect.

Rigidity and bradykinesia are seen more commonly in individuals with drug-induced Parkinsonism than those with Parkinson's disease, where pill-rolling tremor is more common. Drug-induced Parkinsonism tends to be bilateral initially.

The correct answer is: Rigidity



Question **21**

Not answered

Marked out of 1.00

Deep brain stimulation (DBS) is a new physical intervention in psychiatry that is increasingly being used outside of research settings. Which of the following is an indication for DBS?

Select one:

- ☐ Body dysmorphic disorder
- ☐ Bipolar affective disorder
- ☐ OCD
- ☐ Anorexia nervosa
- ☐ Schizophrenia

Your answer is incorrect.

DBS is indicated in Parkinson's disease, which is refractory to or complicated by medical therapy. It can also be used in movement disorders (dystonia or tremors), depression, OCD, Tourette's Syndrome, epilepsy, cluster headache and chronic pain.

The correct answer is: OCD

Question **22**

Not answered

Marked out of 1.00

Abdominal pain is a common presenting symptom amongst children seen in paediatric practice. Which of the following is true regarding abdominal pain in children?

Select one:

- ☐ Abdominal pain is a common symptom of anxiety in children
- ☐ Abdominal pain is usually associated with a persistent disturbance in appetite
- ☐ Abdominal pain related to anxiety in children is usually very well localised to one spot
- ☐ Abdominal pain in children usually responds readily to medical interventions
- ☐ Abdominal pain is usually associated with clear bowel symptoms

Your answer is incorrect.

Abdominal pain in children, whilst it can occasionally be due to significant organic pathology, is usually ill defined and without clearly associated physical symptoms. It is commonly associated with anxiety and worry. Ref; Cunningham NR, et al. Importance of addressing anxiety in youth with functional abdominal pain: suggested guidelines for physicians. Journal of pediatric gastroenterology and nutrition. 2013 May;56(5):469.

The correct answer is: Abdominal pain is a common symptom of anxiety in children

Question **23**

Not answered

Marked out of 1.00

Reading difficulties are relatively common and affected children and adults may need certain adaptations to engage with education and work. What is the male: female ratio of reading difficulties?

Select one:

- ☐ 10 to 1
- ☐ 1 to 3
- ☐ 2 to 1
- ☐ 8 to 1
- ☐ 4 to 1

Your answer is incorrect.

If reading difficulties are defined as 2 standard deviations below mean, then the male: female ratio is 2:1. If 3 standard deviations are used, this increases to 4:1 due to the greater variance in male reading ability. Ref: Arnett AB, et al. Explaining the sex difference in dyslexia. Journal of Child Psychology and Psychiatry. 2017 Jun;58(6):719-27.

The correct answer is: 2 to 1



Question **24**

Not answered

Marked out of 1.00

Anorexia nervosa most commonly affects females and is characterised by body image distortion and behaviours focused upon losing weight, most commonly in terms of dietary restriction but also excessive exercise and purging. When is it most likely to present?

Select one:

- ☐ 20-24 years
- ☐ Before the age of 10 years
- ☐ 25-29 years
- ☐ 11-14 years
- ☐ 15-19 years

Your answer is incorrect.

The average age of onset of anorexia nervosa is 16-17 years.

The correct answer is: 15-19 years



Question 25

Not answered

Marked out of 1.00

Sam Allen is a 10-year-old boy who has been referred to their local CAMHS clinic after it was noted that they were speaking at home but not at school. They are subsequently diagnosed with selective mutism. Which of the following is true regarding selective mutism?

Select one:

- ☐ Usually not associated with other mental disorder
- ☐ It is relatively rare
- ☐ Most common in secondary school
- ☐ More common in males
- ☐ It tends to entirely resolve over several months

Your answer is incorrect.

Selective mutism is relatively rare, occurring in about 0.71% of children. It is highly associated with other anxiety disorders, particularly social phobia. It may be more common in girls than boys and tends to present between the ages of 3-6 years, when a child enters nursery or primary school. Children who recover tend to have ongoing social and talking difficulties.

The correct answer is: It is relatively rare

Question 26

Not answered

Marked out of 1.00

Autism spectrum disorders are characterised by impairments in social interactions and communication and restricted and repetitive interests. They can also be associated with language delay and intellectual impairment. Which of the following is the greatest risk factor for the development of an autism spectrum disorder?

Select one:

- ☐ Maternal smoking
- ☐ Maternal postnatal depression
- ☐ Maternal antidepressant use during pregnancy
- ☐ Receiving the MMR vaccine
- ☐ Low APGAR score

Your answer is incorrect.

A low 5-minute APGAR score is associated with later ASD. Other perinatal risk factors for ASD are: abnormal presentation, umbilical-cord complications, foetal distress, birth injury or trauma, multiple birth, maternal hemorrhage, summer birth, low birth weight, small for gestational age, congenital malformation, feeding difficulties, meconium aspiration, neonatal anemia, ABO or Rh incompatibility, and hyperbilirubinemia. Overall, genetic factors are the greatest known risk factors for ASD. Ref; Modabbernia A, Sandin S, Gross R, Leonard H, Gissler M, Parner ET, Francis R, Carter K, Bresnahan M, Schendel D, Hornig M. Apgar score and risk of autism. European journal of epidemiology. 2018 Oct 5:1-0.

The correct answer is: Low APGAR score

Question **27**

Not answered

Marked out of 1.00

Conduct disorder is a disorder seen in adolescents characterised by repeatedly defiant and transgressing behaviour, often with aggression, rule breaking and other criminal behaviour. Which of the following is the most appropriate first line intervention?

Select one:

- ☐ Fluoxetine
- ☐ Family therapy
- ☐ Interpersonal therapy
- ☐ Parental training
- ☐ CBT

Your answer is incorrect.

Parental psychoeducation and training is most effective in conduct disorder so they can effectively establish and maintain boundaries with the child and enable positive behavioural strategies to be enacted. Achieving behavioural change without clear boundaries and routines is very challenging.

The correct answer is: Parental training



Question **28**

Not answered

Marked out of 1.00

James Borden is a 16-year boy who has been experiencing a moderate depressive episode for the last 6 months. He has received a course of CBT without significant effect on his symptoms. Which of the following is the most appropriate next step for his management?

Select one:

- ☐ Further course of CBT
- ☐ Interpersonal therapy
- ☐ Family therapy
- ☐ Start Fluoxetine
- ☐ Start Citalopram

Your answer is incorrect.

If a child with depression has had an adequate course of CBT without success it is most appropriate to start Fluoxetine as the next step. Fluoxetine is recommended in children, as it is less likely than other antidepressants to cause an increase in suicidal ideation.

The correct answer is: Start Fluoxetine



Question 29

Not answered

Marked out of 1.00

Richard Arbuthnot is an 8-year-old boy who has been referred to the local CAMHS clinic. He is struggling at school due to an inability to sit still or concentrate on any academic tasks. He has also been noted to make a repeated throat clearing noise and twist his head sideways. Which of the following would be the most appropriate treatment for him?

Select one:

- ☐ Risperidone
- ☐ Methylphenidate
- ☐ Atomoxetine
- ☐ Clonidine
- ☐ Fluoxetine

Your answer is incorrect.

This patient is presenting with symptoms consistent with both ADHD and Tourette's syndrome. In these cases, provided the ADHD is not very disabling and Tourette's relatively mild, it is most reasonable to trial Clonidine or guanfacine in the first instance. Methylphenidate would be likely to worsen his vocal and motor tics.

The correct answer is: Clonidine



Question **30**

Not answered

Marked out of 1.00

Which of the following is true with respect to Tourette's disorder?

Select one:

- ☐ Diagnosis involves having both motor and vocal tics for 12 months
- ☐ The treatment of choice is with antipsychotics
- ☐ It is rare for it to co-exist with ASD
- ☐ It can be diagnosed if symptoms start at any age.
- ☐ Diagnosis involves having both motor and vocal tics for 6 months

Your answer is incorrect.

Tourette's Syndrome requires the presence of both motor and vocal tics for at least 12 months. They need not occur at the same time but some tics should be evident throughout this time period. Alpha-2-adrenergic agonists, such as clonidine, are often used first line.

The correct answer is: Diagnosis involves having both motor and vocal tics for 12 months



Question **31**

Not answered

Marked out of 1.00

Some studies have reported a strong correlation between adverse events to SSRIs and their therapeutic efficacy. This has led to the suggestion that the improvement seen in patients allocated to active drug treatment could have been exaggerated as study participants became aware of allocation by experiencing adverse events. To investigate further whether adverse events mediated the relationship between treatment and efficacy, a mediation framework was used by Barth et al. (2016). The result of this analysis is given in the graph. Which of the following conditions must be met in order to establish a mediation effect?



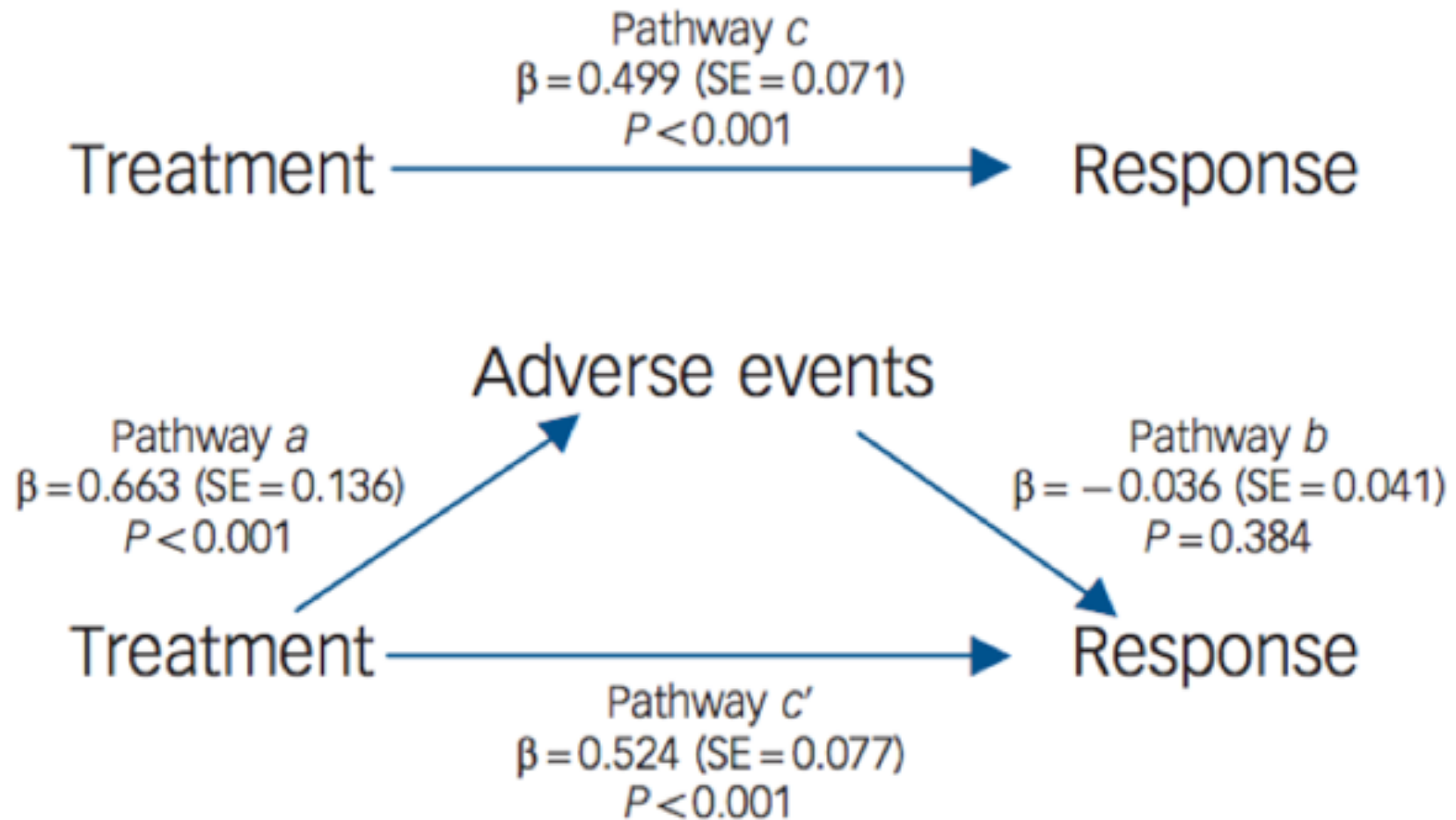


Fig. 3 Mediation model for response, showing the direct pathways (a, b, c) and mediated pathway (c').

Select one:

- ☐ The predictor must relate to the outcome only when controlled for the mediator
- ☐ The relationship between predictor and outcome must be reduced when controlled for the mediator
- ☐ The suspected mediator must not be related to the outcome
- ☐ Predictor must not be related to the suspected mediator
- ☐ Predictor must not be related to the outcome

Your answer is incorrect.

According to Barron & Kenny's mediation model approach mediation effect is said to be present when four conditions are met: 1. The predictor (i.e. treatment) must be significantly related to the outcome (i.e. response). This is shown as pathway c in the graph; 2. The predictor must also be significantly related to the potential mediator (adverse events). This is shown as pathway a in the graph; 3. The mediator must be significantly related to the outcome when the effect of the predictor on the outcome is controlled for; This is shown as pathway b in the graph 4. The relationship between predictor and outcome must be reduced when controlling for the mediator (i.e. pathway c' having effect lower than the pathway c).

The correct answer is: The relationship between predictor and outcome must be reduced when controlled for the mediator



Question **32**

Not answered

Marked out of 1.00

Some studies have reported a strong correlation between adverse events to SSRIs and their therapeutic efficacy. This has led to the suggestion that the improvement seen in patients allocated to active drug treatment could have been exaggerated as study participants became aware of allocation by experiencing adverse events. To investigate further whether adverse events mediated the relationship between treatment and efficacy, a mediation framework was used by Barth et al. (2016). The result of this analysis is given in the graph. If the predictor remains significant when the mediator is controlled for, the mediation is considered to be



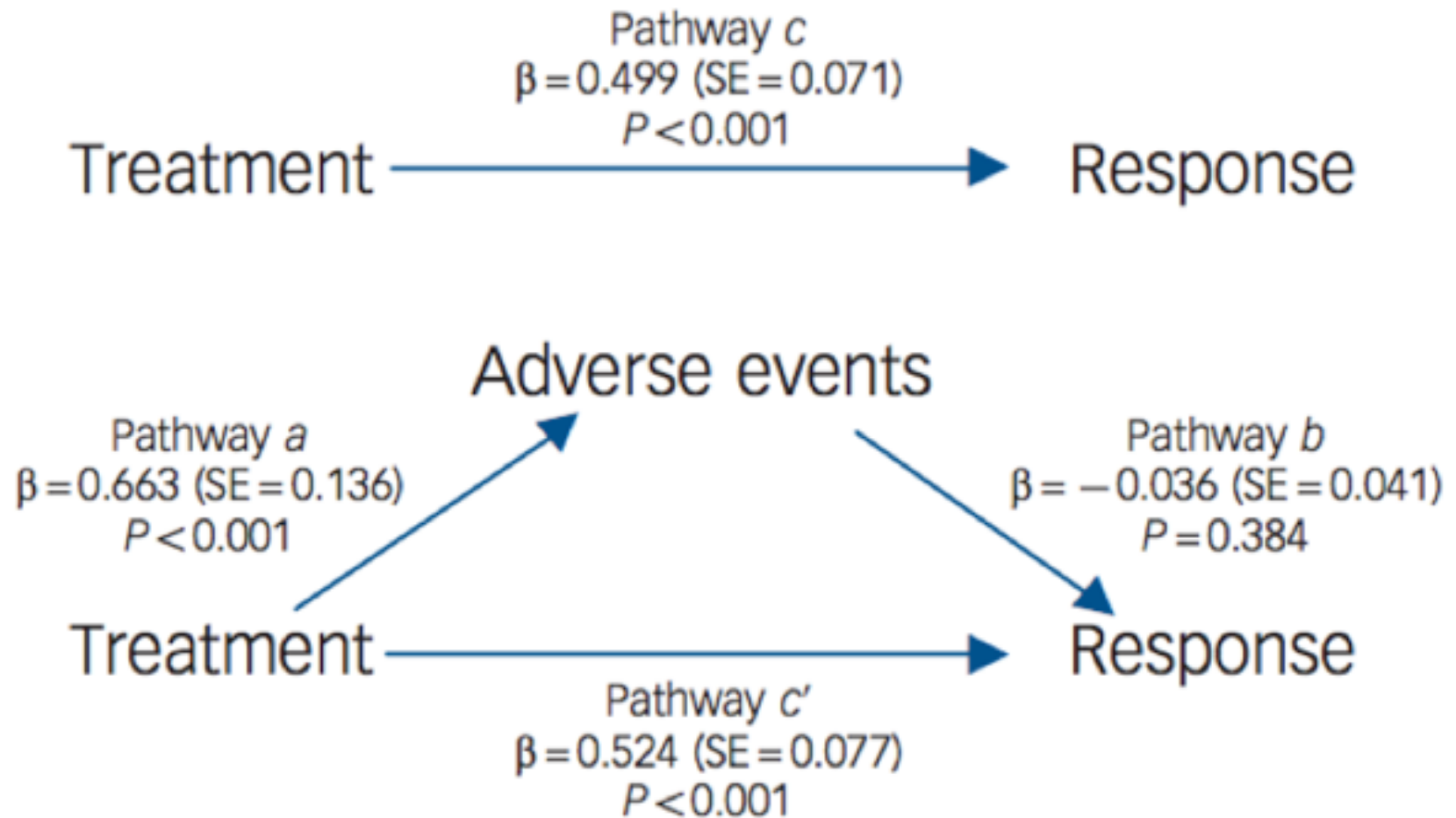


Fig. 3 Mediation model for response, showing the direct pathways (a, b, c) and mediated pathway (c').

Select one:

- ☐ Complete
- ☐ Absent
- ☐ Partial
- ☐ Impartial
- ☐ Insignificant

Your answer is incorrect.

If the predictor remains significant when the mediator is controlled for, the mediation is considered partial. When controlling for the mediator renders the predictor non-significant, mediation is considered complete.

The correct answer is: Partial



Question **33**

Not answered

Marked out of 1.00

Some studies have reported a strong correlation between adverse events to SSRIs and their therapeutic efficacy. This has led to the suggestion that the improvement seen in patients allocated to active drug treatment could have been exaggerated as study participants became aware of allocation by experiencing adverse events. To investigate further whether adverse events mediated the relationship between treatment and efficacy, a mediation framework was used by Barth et al. (2016). The result of this analysis is given in the graph. If controlling for the mediator renders the predictor non-significant, mediation is considered to be



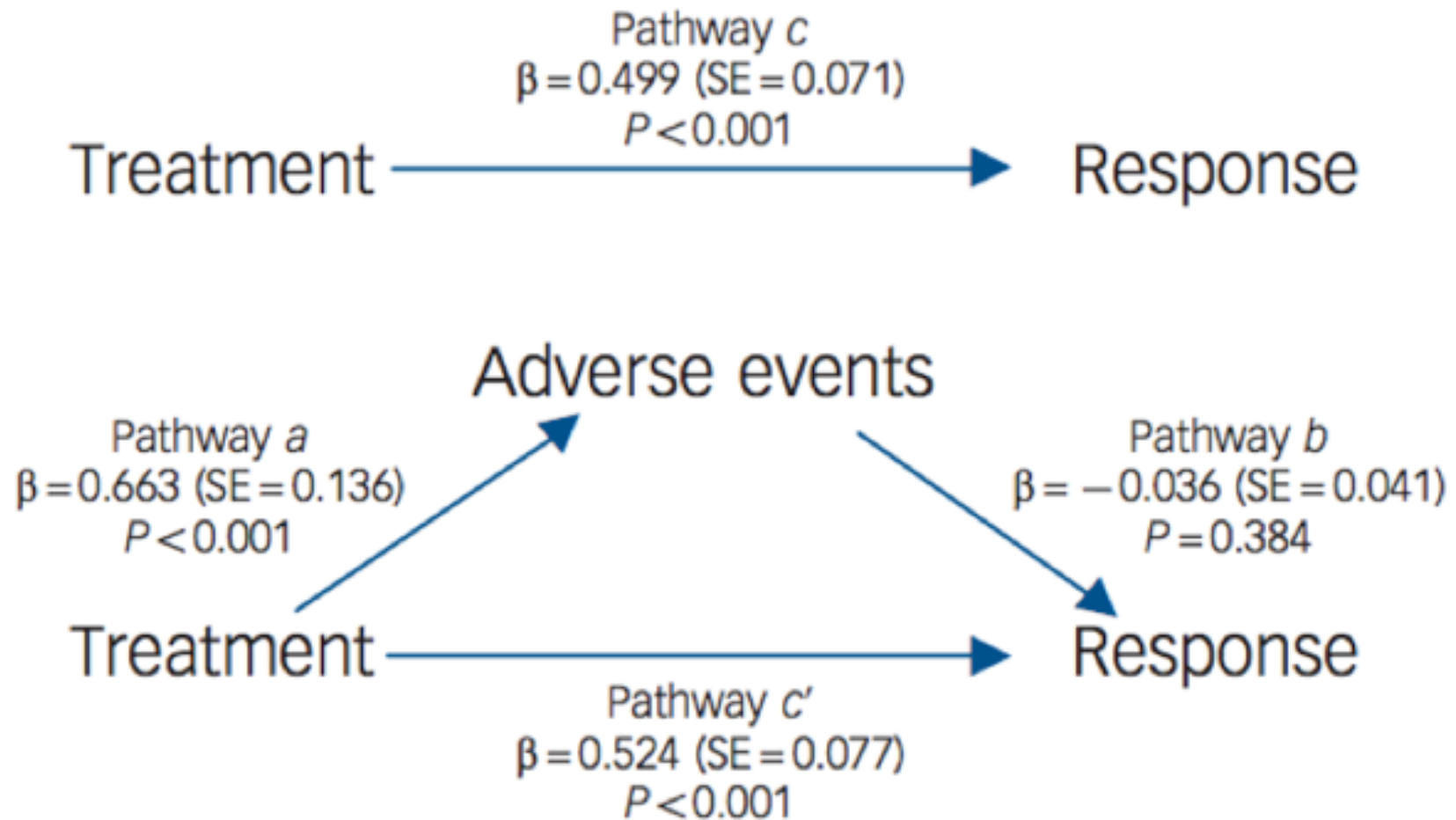


Fig. 3 Mediation model for response, showing the direct pathways (a, b, c) and mediated pathway (c').

Select one:

- ☐ Complete
- ☐ Impartial
- ☐ Partial
- ☐ Insignificant
- ☐ Absent

Your answer is incorrect.

If the predictor remains significant when the mediator is controlled for, the mediation is considered partial. When controlling for the mediator renders the predictor non-significant, mediation is considered complete.

The correct answer is: Complete



Question **34**

Not answered

Marked out of 1.00

Some studies have reported a strong correlation between adverse events to SSRIs and their therapeutic efficacy. This has led to the suggestion that the improvement seen in patients allocated to active drug treatment could have been exaggerated as study participants became aware of allocation by experiencing adverse events. To investigate further whether adverse events mediated the relationship between treatment and efficacy, a mediation framework was used by Barth et al. (2016). The result of this analysis is given in the graph. Based on the graph below, which of the following statement is likely to be correct?



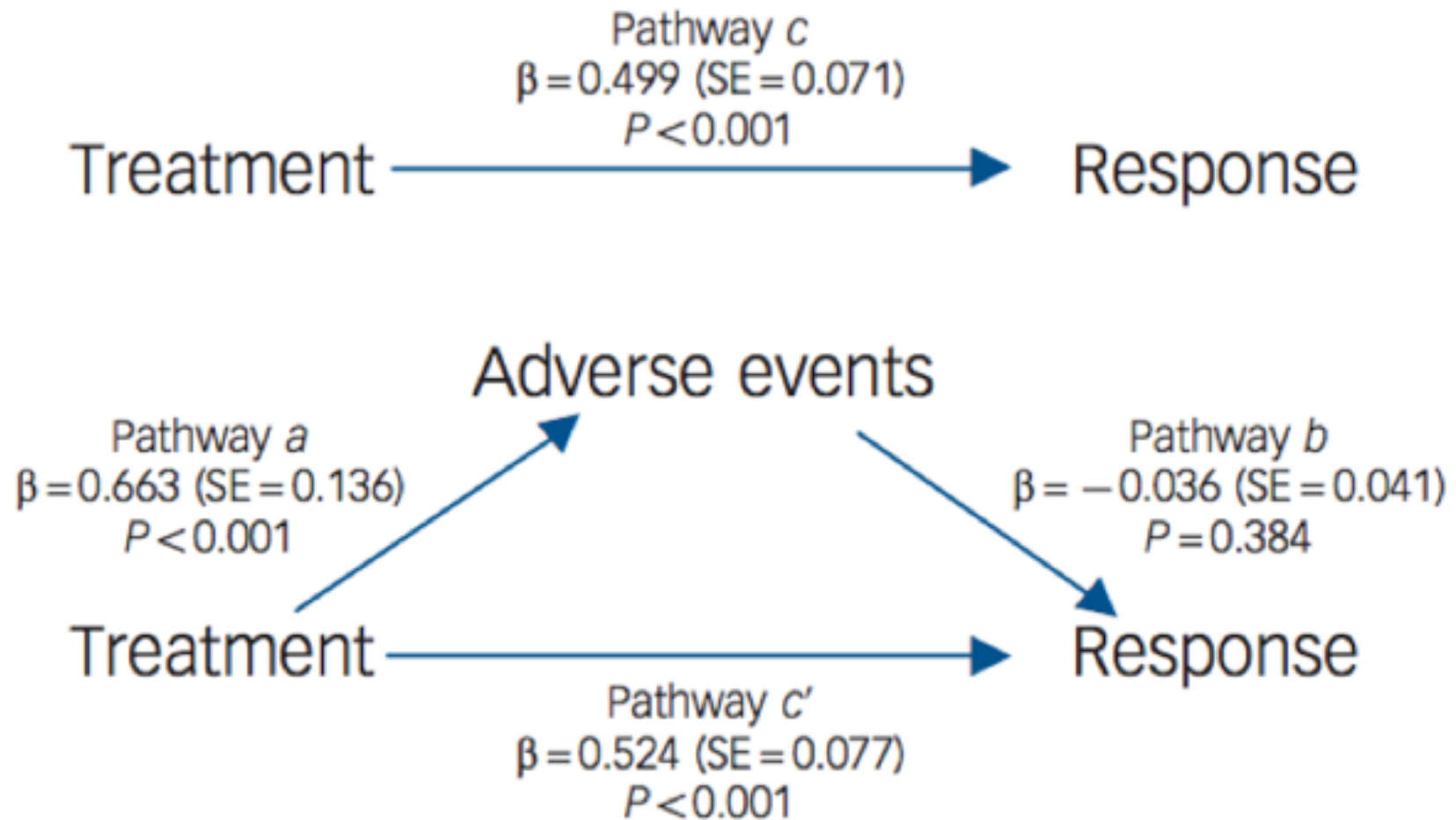


Fig. 3 Mediation model for response, showing the direct pathways (a, b, c) and mediated pathway (c').

Select one:

- ☐ Treatment does not cause adverse effects
- ☐ Adverse effects are proportional to the response
- ☐ Adverse effects do not occur without the response
- ☐ Adverse effects do not mediate response to treatment
- ☐ Treatment does not directly result in therapeutic response

Your answer is incorrect.

Treatment with an SSRI appears to be significantly associated with both response ($b = 0.499$, $SE = 0.071$, $P < 0.001$) and with higher rates of adverse effects ($b = 0.663$, $SE = 0.136$, $P < 0.001$). But there is no significant association between adverse events and outcome per se ($b = 0.036$, $SE = 0.041$, $P = 0.384$). The direct relationship between SSRI treatment and treatment response under control of adverse events (pathway c') is statistically significant ($b = 0.524$, $SE = 0.077$, $P < 0.001$). As one of the conditions for mediation (association of mediator with the outcome under control of the predictor) is not satisfied, we can conclude that there is no evidence for adverse events to mediate the response to SSRI treatment.

The correct answer is: Adverse effects are proportional to the response



Question **35**

Not answered

Marked out of 1.00

Some studies have reported a strong correlation between adverse events to SSRIs and their therapeutic efficacy. This has led to the suggestion that the improvement seen in patients allocated to active drug treatment could have been exaggerated as study participants became aware of allocation by experiencing adverse events. To investigate further whether adverse events mediated the relationship between treatment and efficacy, a mediation framework was used by Barth et al. (2016). The result of this analysis is given in the graph. A Randomised Controlled Trial is planned to study the effect of probiotic use in the treatment of anxiety. The researchers are concerned that patients allocated to the placebo control group could also modify their diet and start taking supplemental probiotics if they come to learn about the experimental diet. This phenomenon is called



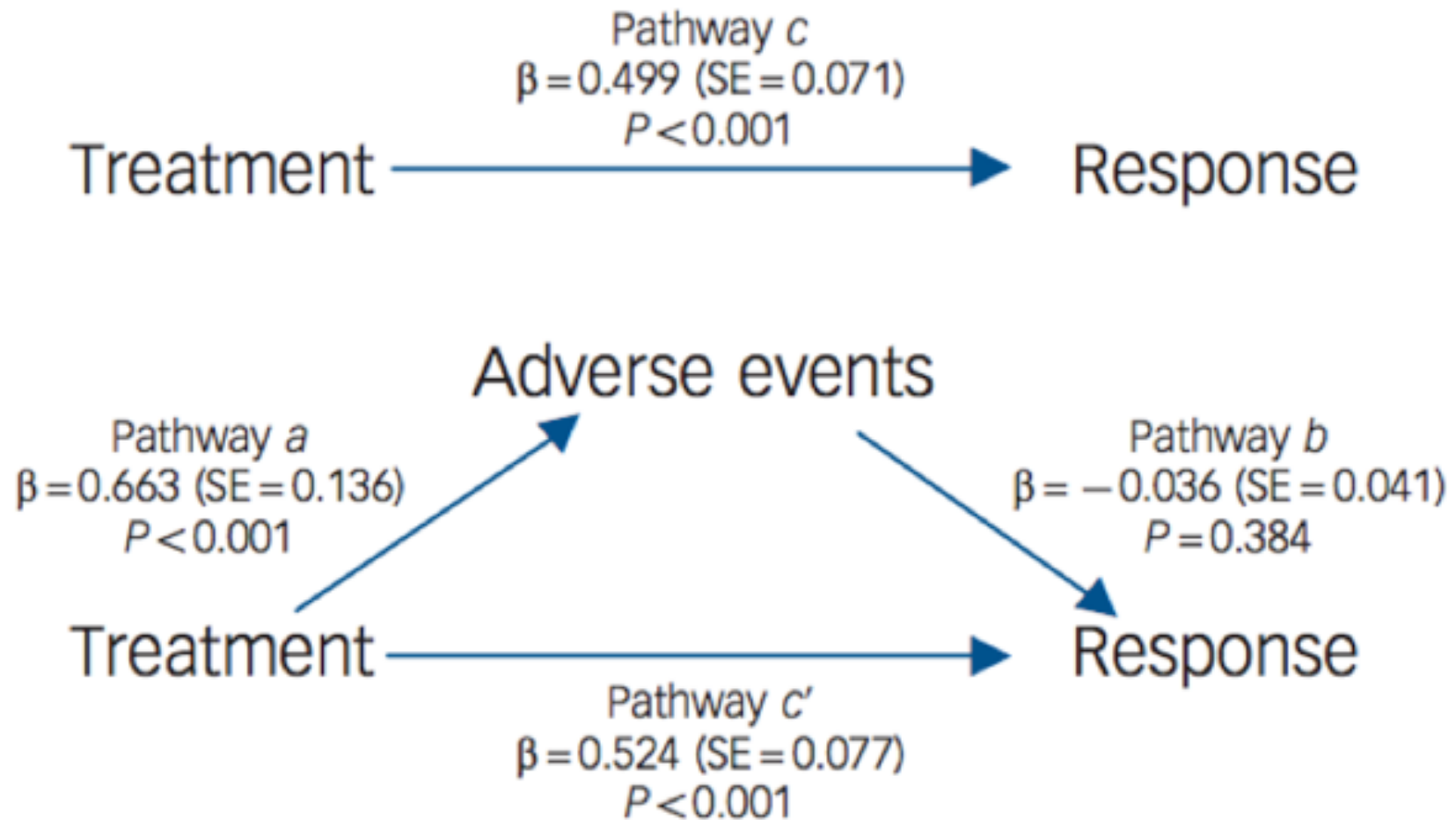


Fig. 3 Mediation model for response, showing the direct pathways (a, b, c) and mediated pathway (c').

Select one:

- ☐ Cross-over
- ☐ Trial centre effect
- ☐ Clustering
- ☐ Contamination
- ☐ Attrition

Your answer is incorrect.

The above phenomenon is best described as contamination of control participants. This can reduce the point estimate of the intervention's effectiveness resulting in a type II error. Ref: Torgerson DJ. Contamination in trials: is cluster randomisation the answer?. Bmj. 2001 Feb 10;322(7282):355-7.

The correct answer is: Contamination



Question **36**

Not answered

Marked out of 1.00

A double-blind Randomised Controlled Trial is planned to study the effect of probiotic use in the treatment of anxiety. The researchers are concerned that patients allocated to the placebo control group could also modify their diet and start taking supplemental probiotics if they come to learn about the experimental diet. Which of the following methodological approach can reduce or avoid this effect?

Select one:

- ☐ Run-on phase
- ☐ Simple randomisation
- ☐ Cluster randomisation
- ☐ Allocation concealment
- ☐ Blinding

Your answer is incorrect.

Cluster randomisation allows entire groups of subjects that are geographically (or otherwise) clustered to be allocated to different treatments. This can potentially reduce the contact between control and active intervention arms and thus reduce treatment contamination. Ref: Campbell MK, Grimshaw JM. Cluster randomised trials: time for improvement. BMJ. 1998;317:1171-1172.

The correct answer is: Cluster randomisation

Question **37**

Not answered

Marked out of 1.00

A double-blind Randomised Controlled Trial is planned to study the effect of motivational counselling against alcohol use on the treatment of depression in patients older than 35 years of age. The researchers recruit various General Practice surgeries for this purpose. The intervention is allocated randomly to the General Practice surgeries, and once allocated, all patients recruited following their informed consent at a centre are provided the same intervention (either counselling or place on a waiting-list). Which of the following bias is likely to result from this approach?

Select one:

- ☐ Cross-over effect
- ☐ Misclassification effect
- ☐ Attrition effect
- ☐ Contamination effect
- ☐ Trial-centre effect

Your answer is incorrect.

This example describes a cluster-randomised trials where centres (not individuals) are randomised to treatment arms. This is called clustering or trial-centre effect, defined as when observations are grouped together based upon common attributes. Such effect can also be seen in multicentre trials where patients are clustered within centres, and trials where multiple therapists or surgeons are allocated to be the providers of care. Clustering can lead to incorrect (i.e. too small) estimates of the precision and therefore incorrect type I error rates (i.e. p values of the point estimate). Ref: Kahan BC, Morris TP. Assessing potential sources of clustering in individually randomised trials. BMC medical research methodology. 2013 Dec;13(1):58.

The correct answer is: Trial-centre effect

Question **38**

Not answered

Marked out of 1.00

Which of the following can be used to reduce trial-centre effect in RCTs involving various general practice clusters?

Select one:

- ☐ Simple randomisation
- ☐ Systematic sampling of trial centres
- ☐ Open-label trial
- ☐ Cluster randomisation
- ☐ Non random sampling

Your answer is incorrect.

Treatment assignments between patients in the same cluster will be correlated if patients in certain clusters are more likely to be in a certain treatment group. This correlation is 1 in cluster RCTs wherein all patients recruited following their informed consent at a centre are provided the same intervention. Simple randomisation where all patients are randomised independently leads to a zero-correlation among treatment assignments. Ref: Kahan BC, Morris TP. Assessing potential sources of clustering in individually randomised trials. BMC medical research methodology. 2013 Dec;13(1):58.

The correct answer is: Simple randomisation

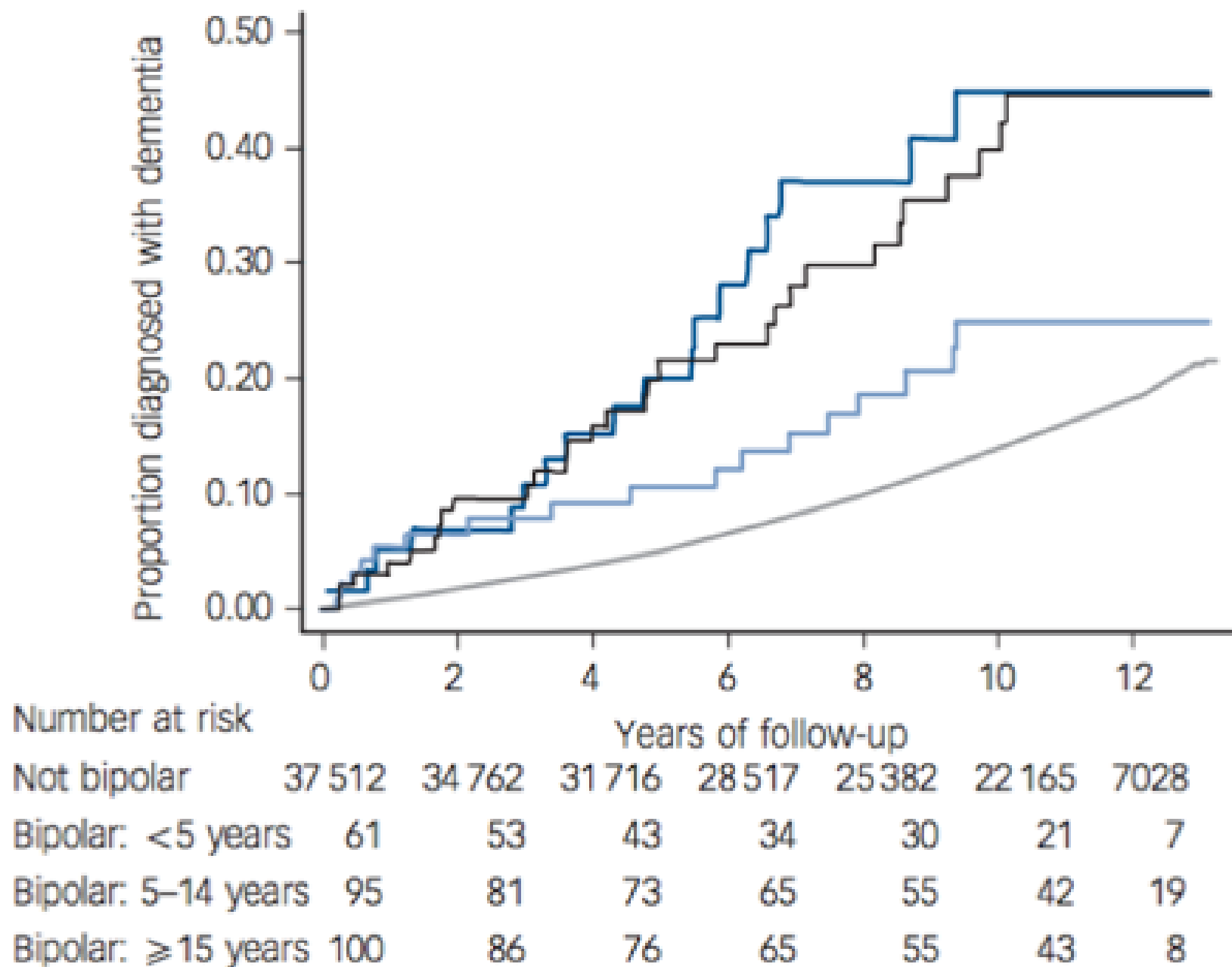
Question **39**

Not answered

Marked out of 1.00

To investigate the 13-year risk of dementia and death in older adults with bipolar disorder, a cohort study of 37 768 men aged 65-85 years was conducted. The outcomes dementia and death were traced from the data collected for The Health in Men Study (HIMS), a cohort study of a community-representative sample of older Western Australian men that started in 1996. Follow-up data were censored on 30 June 2009, which is the last date for which data on cause-specific mortality are available. A total study sample of 37 768 older men who were alive and free of dementia by 2 April 1996 were included in the investigation. Bipolar disorder was associated with increased adjusted hazard ratio (HR) of dementia (HR = 2.30, 95% CI 1.80-2.94). Bipolar disorder was also associated with increased mortality (HR = 1.51, 95% CI 1.28-1.77). See the graph below: With respect to the graph below, the rates of dementia appear to be very low at the beginning of the study. This is due to





Select one:

- ☐ Right censor
- ☐ Underdiagnosis
- ☐ Delay in diagnosis
- ☐ Left censoring
- ☐ Attrition

Fig. 2 Age-adjusted cumulative hazard of dementia according to the time lived with the diagnosis of bipolar disorder.

Not bipolar (grey line – reference), <5 years (dark blue line; hazard ratio (HR) 3.23, 95% CI 2.03–5.14), 5–14 years (pale blue line; HR = 1.71, 95% CI 1.06–2.76), ≥15 years (black; HR = 3.09, 95% CI 2.16–4.43). The respective age-adjusted sub-hazard

Your answer is incorrect.

Right censoring occurs when a subject leaves the study before the event of interest (drop-out) or if the study ends before the event has occurred. Right censoring is very common. Left censoring occurs if the event of interest (dementia) has already occurred before enrolment. Right truncation occurs when the entire study population has already experienced the event of interest (i.e. dementia in this case). This can be seen in historical surveys of patients who have the outcome of interest from a registry (e.g. the use of a dementia registry for this question). Left truncation occurs when the subjects have been at risk before entering the study. In this study, left truncation is negligible as the risk of dementia is smaller before the age of 65. Ref: www.stats.ox.ac.uk/~mlunn/lecturenotes1.pdf

The correct answer is: Left censoring



Question **40**

Not answered

Marked out of 1.00

To investigate the 13-year risk of dementia and death in older adults with bipolar disorder, a cohort study of 37 768 men aged 65-85 years was conducted. The outcomes dementia and death were traced from the data collected for The Health in Men Study (HIMS), a cohort study of a community-representative sample of older Western Australian men that started in 1996. Follow-up data were censored on 30 June 2009, which is the last date for which data on cause-specific mortality are available. A total study sample of 37 768 older men who were alive and free of dementia by 2 April 1996 were included in the investigation. Bipolar disorder was associated with increased adjusted hazard ratio (HR) of dementia (HR = 2.30, 95% CI 1.80-2.94). Bipolar disorder was also associated with increased mortality (HR = 1.51, 95% CI 1.28-1.77). See the graph below: Which of the 2 groups of patients in the survival graph are most closely related to each other in terms of the risk of a diagnosis of dementia?



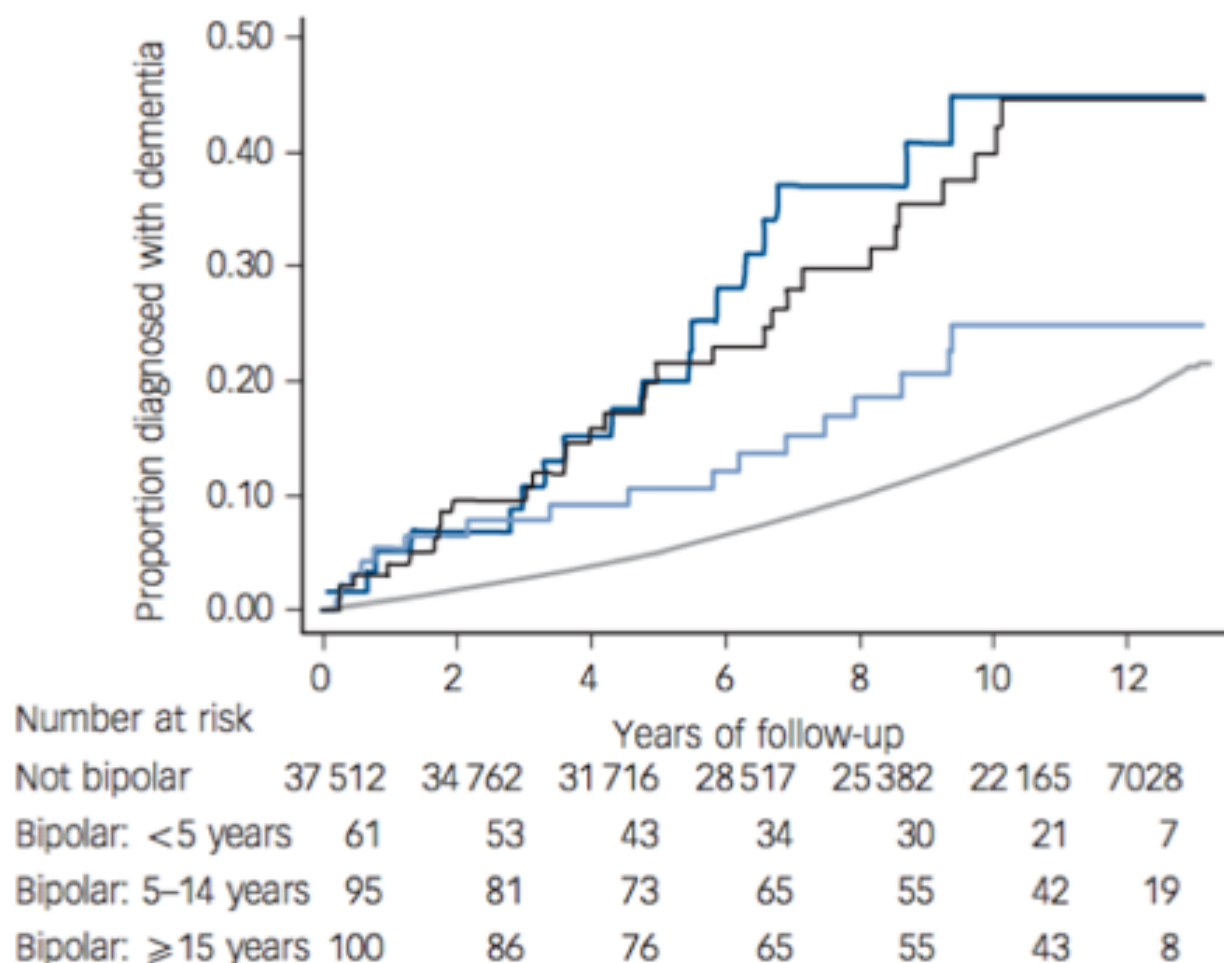


Fig. 2 Age-adjusted cumulative hazard of dementia according to the time lived with the diagnosis of bipolar disorder.

Not bipolar (grey line – reference), <5 years (dark blue line; hazard ratio (HR) 3.23, 95% CI 2.03–5.14), 5–14 years (pale blue line; HR = 1.71, 95% CI 1.06–2.76), ≥15 years (black; HR = 3.09, 95% CI 2.16–4.43). The respective age-adjusted sub-hazard ratios of dementia were 2.69 (95% CI 1.62–4.48), 1.41 (95% CI 0.86–2.31) and 2.68 (95% CI 1.81–3.95) with death used as a competing risk.

Select one:

- ☐ Patients with ≥ 15 years if illness and the non bipolar group
- ☐ Non-bipolar group and < 5 years of illness
- ☐ Patients with 5-14 years and < 5 years of illness
- ☐ Patients with 5-14 years and ≥ 15 years if illness
- ☐ Patients with ≥ 15 years and < 5 years of illness

Your answer is incorrect.

These results displayed in the figure suggest that risk of dementia was similarly high among men with recent (< 5 years, dark blue) and long-standing history of bipolar disorder (≥ 15 years, black line) compared with participants without bipolar disorder.

The correct answer is: Patients with ≥ 15 years and < 5 years of illness



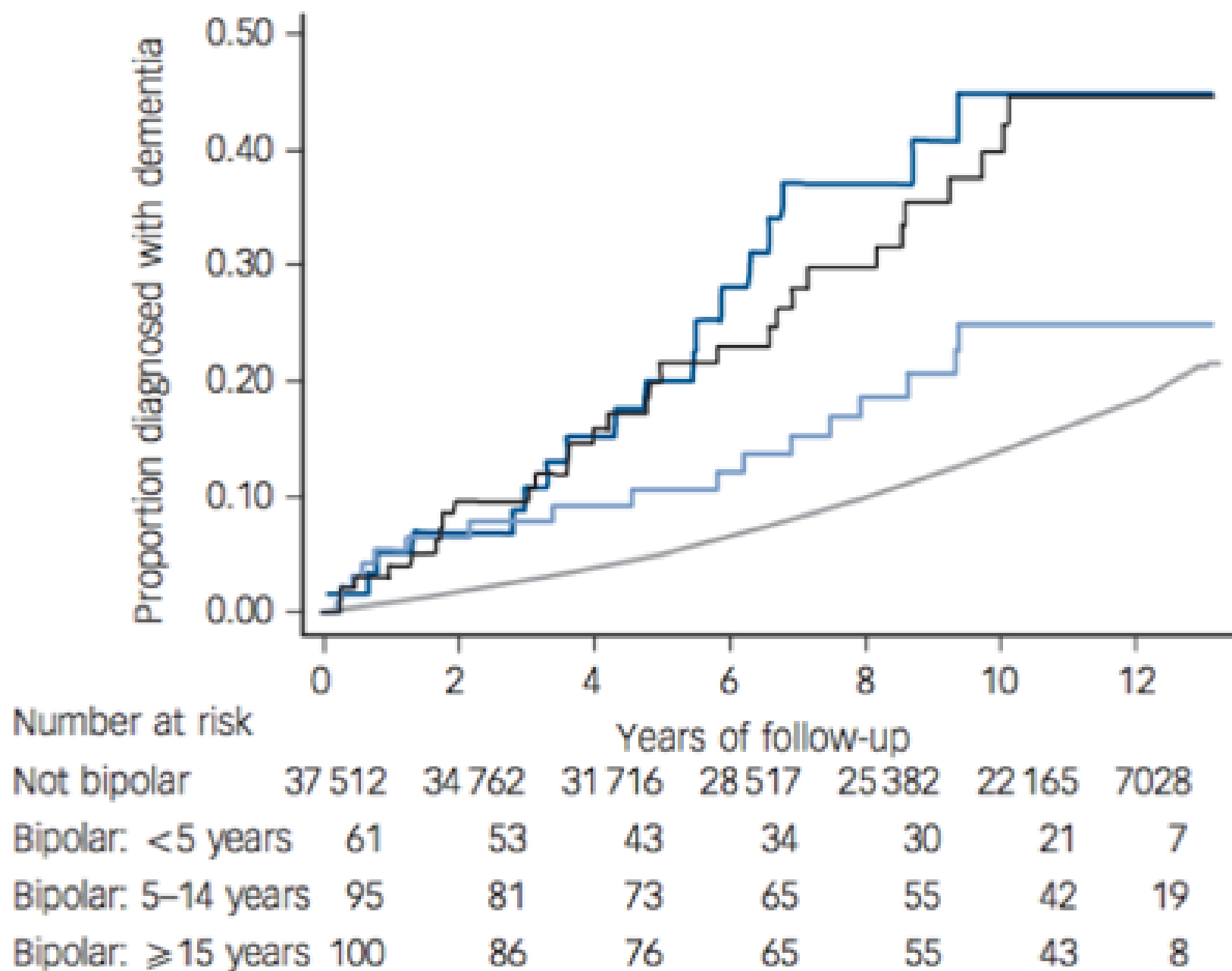
Question **41**

Not answered

Marked out of 1.00

To investigate the 13-year risk of dementia and death in older adults with bipolar disorder, a cohort study of 37 768 men aged 65–85 years was conducted. The outcomes dementia and death were traced from the data collected for The Health in Men Study (HIMS), a cohort study of a community-representative sample of older Western Australian men that started in 1996. Follow-up data were censored on 30 June 2009, which is the last date for which data on cause-specific mortality are available. A total study sample of 37 768 older men who were alive and free of dementia by 2 April 1996 were included in the investigation. Bipolar disorder was associated with increased adjusted hazard ratio (HR) of dementia (HR = 2.30, 95% CI 1.80–2.94). Bipolar disorder was also associated with increased mortality (HR = 1.51, 95% CI 1.28–1.77). See the attached graph below: The best description of the association between risk of dementia and the duration of living with a diagnosis of bipolar disorder is





Select one:

- ☐ no notable relationship
- ☐ non-linear
- ☐ parametric
- ☐ dose dependent
- ☐ linear

Fig. 2 Age-adjusted cumulative hazard of dementia according to the time lived with the diagnosis of bipolar disorder.

Not bipolar (grey line – reference), <5 years (dark blue line; hazard ratio (HR) 3.23, 95% CI 2.03–5.14), 5–14 years (pale blue line; HR = 1.71, 95% CI 1.06–2.76), ≥15 years (black; HR = 3.09, 95% CI 2.16–4.43). The respective age-adjusted sub-hazard

Your answer is incorrect.

The authors did find that the risk of dementia varied according to the duration of bipolar disorder, with dementia being more frequent among men with <5 years history of bipolar disorder or with >15 years of illness. This relationship is not linearly dose dependent, as patients with 5-14 years of illness have lower risk than the group with <5 years, even though they are at higher risk than the reference group with no bipolar disorder.

The correct answer is: non-linear



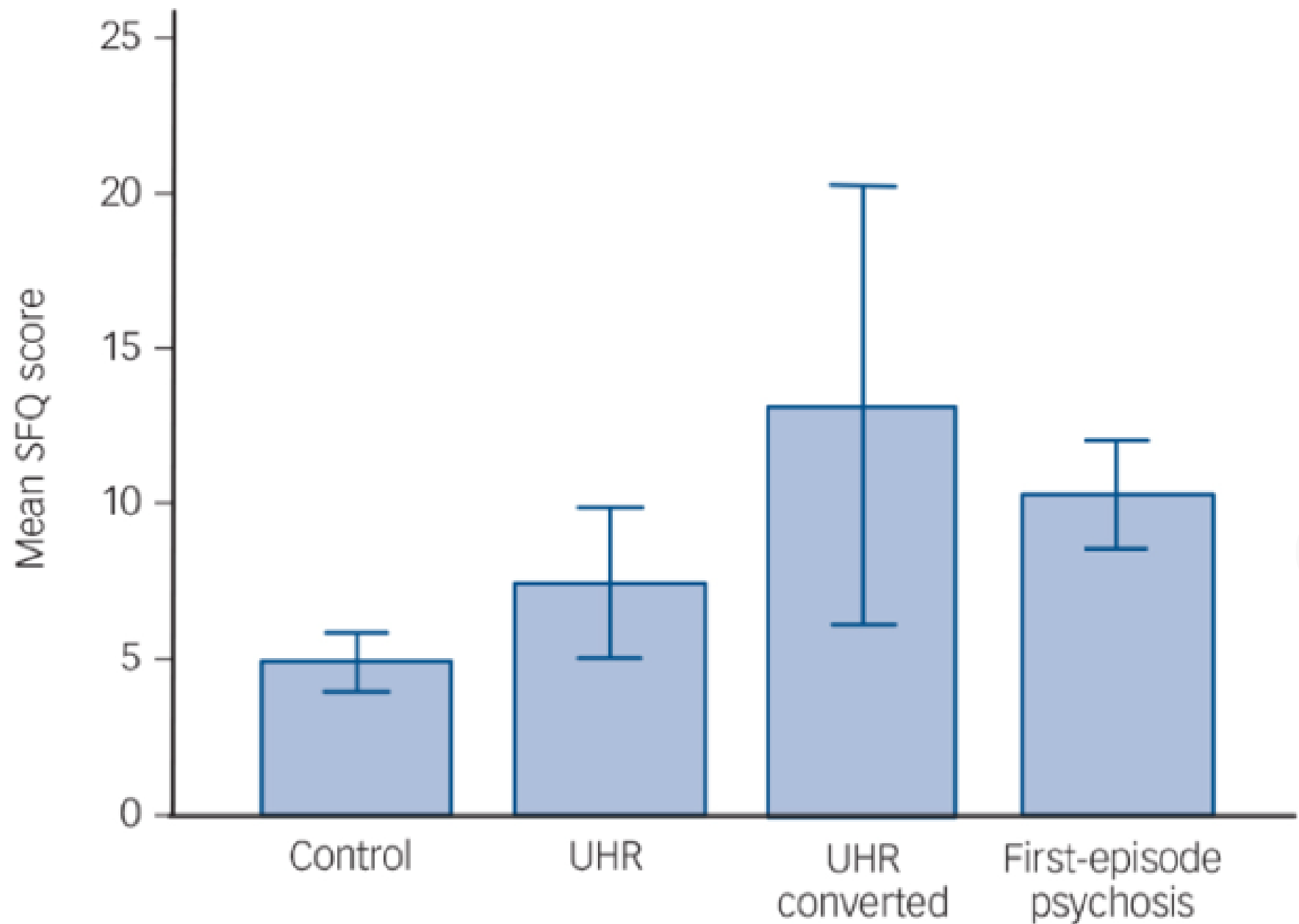
Question **42**

Not answered

Marked out of 1.00

To compare sexual function in people at different stages of a psychotic illness and healthy volunteers, sexual function was assessed in an untreated ultra-high risk UHR group (n = 31), a group with first-episode psychosis (n = 37) and a matched control group of healthy volunteers (n = 56) using the Sexual Function Questionnaire. Higher Sexual Function Questionnaire (SFQ) scores indicate poorer sexual function. The group at ultra-high risk (UHR) of psychosis was divided into those who made the transition to psychosis (UHR converted) and those who did not (UHR) for further analysis. Please see the attached graph to answer the questions below. Ref: Marques, T., et al. (2012). Sexual dysfunction in people with prodromal or first-episode psychosis. *British Journal of Psychiatry*, 201(2), 131-136. The graph is called





Select one:

- ☐ Box Whisker plot
- ☐ Frequency plot
- ☐ Bar plot
- ☐ Histogram
- ☐ Manhattan plot

Your answer is incorrect.

The shown figure is a bar plot, with different groups having their mean values and dispersion around the mean displayed.

The correct answer is: Bar plot



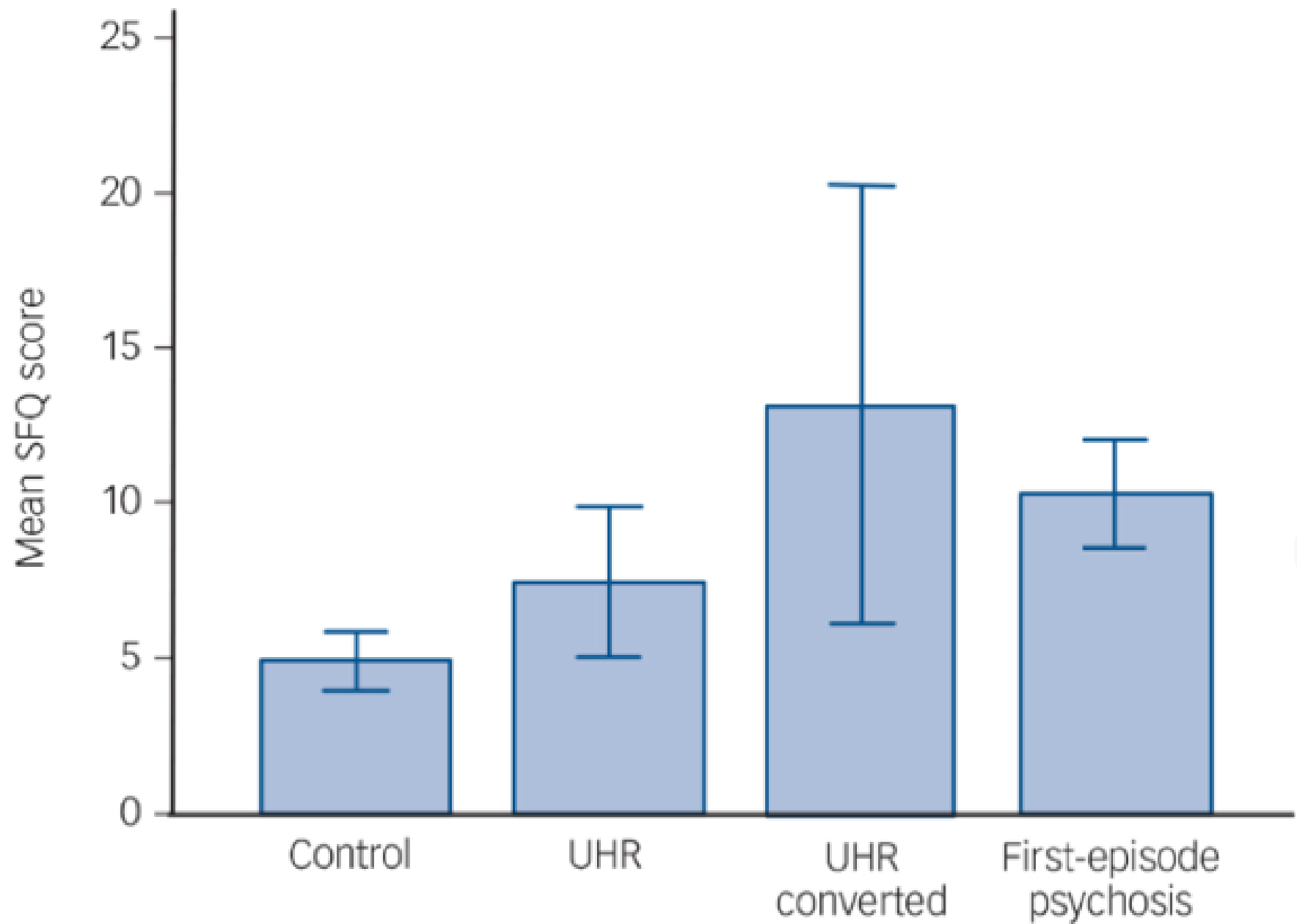
Question **43**

Not answered

Marked out of 1.00

To compare sexual function in people at different stages of a psychotic illness and healthy volunteers, sexual function was assessed in an untreated ultra-high risk UHR group (n = 31), a group with first-episode psychosis (n = 37) and a matched control group of healthy volunteers (n = 56) using the Sexual Function Questionnaire. Higher Sexual Function Questionnaire (SFQ) scores indicate poorer sexual function. The group at ultra-high risk (UHR) of psychosis was divided into those who made the transition to psychosis (UHR converted) and those who did not (UHR) for further analysis. Please see the graph to answer the questions below. Ref: Marques, T., et al. (2012). Sexual dysfunction in people with prodromal or first-episode psychosis. *British Journal of Psychiatry*, 201(2), 131-136. If the error bars indicate 95% confidence intervals, which of the following conclusion can be drawn from the figure?





Select one:

- ☐ UHR subjects that later convert have the best sexual function
- ☐ Control subjects have the lowest sexual function.
- ☐ Sexual dysfunction is present prior to onset of psychosis
- ☐ Sexual function is not affected in psychosis compared to control subjects
- ☐ Sexual dysfunction in psychosis is related to antipsychotic medication

Your answer is incorrect.

These results suggest that sexual dysfunction is likely to be seen prior to onset of psychosis, suggesting some intrinsic relationship not always mediated by the prolactin-raising properties of antipsychotic medications.

The correct answer is: Sexual dysfunction is present prior to onset of psychosis



Question 44

Not answered

Marked out of 1.00

Concord grape juice is said to improve memory. To investigate this further, 230 ml purple grape juice or sugar-matched control was administered to 20 elderly subjects living in Newcastle-upon-Tyne and surrounding areas. The subjects received both interventions in a double-blind manner after 6-7 days of washout period in a random sequence. Computerised measures of episodic memory, working memory, attention and mood were completed at baseline and following a 20-min absorption period. Purple grape juice significantly improved reaction time on a composite attention measure ($p = 0.047$) and increased calm ratings ($p = 0.046$) when compared to placebo. Order effects also indicated an enduring positive effect on pre-dose memory reaction time ($p = 0.018$) and post-dose calm ratings ($p = 0.019$) when purple grape was consumed first. No significant effects of juice were observed on memory measures. Which of the following best describes the above study design?

Select one:

- ☐ Parallel RCT
- ☐ Cross-over RCT
- ☐ Run-on RCT
- ☐ Factorial RCT
- ☐ Open-label RCT

Your answer is incorrect.

This is a randomised, placebo-controlled, double blind crossover study.

The correct answer is: Cross-over RCT

Question 45

Not answered

Marked out of 1.00

Concord grape juice is said to improve memory. To investigate this further, 230 ml purple grape juice or sugar-matched control was administered to 20 elderly subjects living in Newcastle-upon-Tyne and surrounding areas. The subjects received both interventions in a double-blind manner after 6-7 days of washout period in a random sequence. Computerised measures of episodic memory, working memory, attention and mood were completed at baseline and following a 20-min absorption period. Purple grape juice significantly improved reaction time on a composite attention measure ($p = 0.047$) and increased calm ratings ($p = 0.046$) when compared to placebo. Order effects also indicated an enduring positive effect on pre-dose memory reaction time ($p = 0.018$) and post-dose calm ratings ($p = 0.019$) when purple grape was consumed first. No significant effects of juice were observed on memory measures. Which of the following limitation is partially mitigated by the crossover design?

Select one:

- ☐ Short duration of observation
- ☐ Failure of blinding
- ☐ Small sample size
- ☐ Lack of active control group
- ☐ Lack of generalisability

Your answer is incorrect.

A crossover design has several advantages over a parallel RCT. As the subjects in both arms are essentially the same, the comparability of study and control groups with regard to confounding variables (e.g., age and sex) is greatly enhanced. In addition, trials with a crossover design require lower sample sizes than parallel-group trials to meet the same criteria in terms of type I and type II error risks.

The correct answer is: Small sample size

Question 46

Not answered

Marked out of 1.00

To assess whether maternal use of selective serotonin reuptake inhibitors (SSRIs) increases the risk of persistent pulmonary hypertension in the newborn, a population based cohort study using data from the national health registers from Denmark, Finland, Iceland, Norway, and Sweden, between 1996-2007. Comparisons were made between infants exposed and not exposed to SSRIs. Around 30 000 women had used SSRIs during pregnancy and 11 014 had been dispensed an SSRI later than gestational week 20. Exposure to SSRIs in late pregnancy was associated with an increased risk of persistent pulmonary hypertension in the newborn: 33 of 11 014 exposed infants (absolute risk 3 per 1000 liveborn infants compared with the background incidence of 1.2 per 1000); adjusted odds ratio 2.1 (95% confidence interval 1.5 to 3.0). The increased risks of persistent pulmonary hypertension in the newborn for each of the specific SSRIs (sertraline, citalopram, paroxetine, and fluoxetine) were of similar magnitude. Filling a prescription with SSRIs before gestational week 8 yielded slightly increased risks: adjusted odds ratio 1.4 (95% confidence interval 1.0 to 2.0). Ref: Kieler H, et al. Selective serotonin reuptake inhibitors during pregnancy and risk of persistent pulmonary hypertension in the newborn: population based cohort study from the five Nordic countries. BMJ. 2012 Jan 12;344:d8012. Which of the following is a valid conclusion from this study?

Select one:

- ☐ The use of SSRIs in early pregnancy increases the risk of PPHN
- ☐ The use of SSRIs in late pregnancy reduces the risk of PPHN
- ☐ The use of SSRIs in pregnancy is not advisable
- ☐ The use of SSRIs in late pregnancy increases the risk of PPHN
- ☐ The use of SSRIs in early pregnancy reduces the risk of PPHN

Your answer is incorrect.

The results suggest that the absolute risk of persistent pulmonary hypertension of the newborn is low (1.2 per 1000), but use of SSRIs in late pregnancy increases that risk more than twofold (by a factor of 2.1). The increased risk appears to be a class effect, affecting all SSRIs. In early pregnancy the increased risk is not statistically significant.

The correct answer is: The use of SSRIs in late pregnancy increases the risk of PPHN

Question 47

Not answered

Marked out of 1.00

To assess whether maternal use of selective serotonin reuptake inhibitors (SSRIs) increases the risk of persistent pulmonary hypertension in the newborn, a population based cohort study using data from the national health registers from Denmark, Finland, Iceland, Norway, and Sweden, between 1996-2007. Comparisons were made between infants exposed and not exposed to SSRIs. Around 30 000 women had used SSRIs during pregnancy and 11 014 had been dispensed an SSRI later than gestational week 20. Exposure to SSRIs in late pregnancy was associated with an increased risk of persistent pulmonary hypertension in the newborn: 33 of 11 014 exposed infants (absolute risk 3 per 1000 liveborn infants compared with the background incidence of 1.2 per 1000); adjusted odds ratio 2.1 (95% confidence interval 1.5 to 3.0). The increased risks of persistent pulmonary hypertension in the newborn for each of the specific SSRIs (sertraline, citalopram, paroxetine, and fluoxetine) were of similar magnitude. Filling a prescription with SSRIs before gestational week 8 yielded slightly increased risks: adjusted odds ratio 1.4 (95% confidence interval 1.0 to 2.0). Ref: Kieler H, et al. Selective serotonin reuptake inhibitors during pregnancy and risk of persistent pulmonary hypertension in the newborn: population based cohort study from the five Nordic countries. BMJ. 2012 Jan 12;344:d8012. Based on this study what would you tell a pregnant patient about the risk of using paroxetine in pregnancy?

Select one:

- ☐ There is no great risk of PPHN. The patient should stay on medication
- ☐ There is a potential association between Paroxetine in late pregnancy and PPHN
- ☐ There is an increased risk of PPHN. The patient must be strongly advised to stop this treatment
- ☐ Paroxetine causes PPHN across all stages of pregnancy
- ☐ All antidepressants cause PPHN and should be stopped in pregnancy

Your answer is incorrect.

We cannot directly infer the risk of other non-SSRI antidepressants from the data presented to us. So statement A is incorrect. We cannot make affirmative causal inferences, as this is an observational study. Also, the risk appears to be different across the 2 stages of pregnancy. So statement B is incorrect as well. Statement C is the most accurate.

The correct answer is: There is a potential association between Paroxetine in late pregnancy and PPHN

Question 48

Not answered

Marked out of 1.00

Harwood et al. analysed the data from California Drug and Alcohol Treatment Assessment of 1992 to investigate the economic benefits of methadone use in women and men (Table from Cartwright WS. Cost-benefit analysis of drug treatment services: review of the literature. The Journal of Mental Health Policy and Economics. 2000 Mar;3(1):11-26.). Women were 38% of the total treatment population and averaged \$1381 per patient in treatment cost. From the information presented, which of the following is a valid conclusion that can be derived? Using methadone for detoxification is:

Table 16. The California Drug and Alcohol Treatment Assessment: women and men

Program type	Length of stay (days)	Treatment costs per episode (\$)	Total benefits (\$)	Benefit-cost ratio
Residential				
Women	72	4405	10 744	2.4
Men	67	4391	27 093	6.2
Social model				
Women	74	2712	6509	4.0
Men	81	2794	12 435	4.5
Outpatient drug free				
Women	187	990	2853	7.4
Men	122	959	13 302	13.9
Methadone detoxification				
Women	64	405	1206	-3.7
Men	57	395	7057	17.9
Methadone maintenance				
Women	304	2325	10 833	5.3
Men	338	2115	11 637	5.5

Source: Harwood HJ, Fountain D, Carothers S, Gerstein D, Johnson R. 1998.³³

Select one:

- ☐ Good value for both men and women
- ☐ Good value for men but not for women
- ☐ No value for either sex
- ☐ Good value for women but not for men
- ☐ No comment can be made on the value of methadone

Your answer is incorrect.

CBA or cost-benefit analysis is an economic evaluation of the costs of an intervention compared to the benefits of the intervention, with both costs and benefits measured in the same monetary units. Comparing the economic expenditure imposed by participants before treatment with the expenditures incurred due to and after the treatment helps us to assess the 'benefits' of a treatment. Costs that are avoided (by intervening) results in benefits. Benefits take on a negative value if economic costs are greater during or after treatment compared to before treatment. In the table provided, the benefits from methadone detoxification are better for men than women.

The correct answer is: Good value for men but not for women



Question 49

Not answered

Marked out of 1.00

A new Machine Learning Algorithm (MLA) is compared to the validated NAPLS-2 criteria for predicting conversion to psychosis in subjects that are identified to have a Clinical High Risk (CHR) status. 24 of 400 CHR subjects are predicted to convert in the next 2 years, according to the NAPLS-2 criteria. The MLA identifies 40 of the 400 as potential converters to psychosis, with 16 subjects being identified as converters by both the NAPLS-2 criteria and the MLA. What is the predictive value of a negative MLA test in this sample of subjects in the CHR state?

Select one:

- ☐ 56%
- ☐ 78%
- ☐ 98%
- ☐ 88%
- ☐ 67%

Your answer is incorrect.

To answer this question, first construct a 2X2 table. The values provided in the question can be used to fill the cells as below. NPV = true negative (352) divided by total negatives (360) = $352/360 = 97.7\%$

	NAPLS-2 converters	NAPLS-2 non-converters	<u>total</u>
MLA +	16	24	40
MLA -	8	352	360
<u>total</u>	24	376	400

The correct answer is: 98%

Question 50

Not answered

Marked out of 1.00

Expectations and preconceptions can potentially taint the process of qualitative research. Which of the following methods is used in qualitative research to mitigate the potentially deleterious effects of researcher preconceptions?

Select one:

- ☐ Publishing the protocol
- ☐ Triangulation
- ☐ Bracketing
- ☐ Focus groups
- ☐ Pre-registering the study

Your answer is incorrect.

Bracketing includes a set of methods that stem from a researcher's commitment to surfacing his or her preconceptions both before and during the qualitative research process. The various bracketing approaches include methods such as (1) keeping a reflexive journal begun prior to defining the research question (2) engaging in interviews with an outside source to uncover and bring into awareness preconceptions and biases (3) writing memos throughout data collection and analysis as a means of examining and reflecting upon the researcher's engagement with the data. Ref: Excerpts from Tufford L, Newman P. Bracketing in qualitative research. Qualitative social work. 2012 Jan;11(1):80-96.

The correct answer is: Bracketing

Question 51

Not answered

Marked out of 1.00

Stalking is a relatively common phenomenon and often leads to the victims experiencing significant distress due to disruption to their life and fear of being physically harmed. Which of the following types of stalker poses the greatest risk of sexual violence?

Select one:

- ☐ Rejected
- ☐ Resentful
- ☐ Incompetent suitors
- ☐ Intimacy seekers
- ☐ Predatory

Your answer is incorrect.

In a landmark study of stalker conducted by Mullen et al (1999), 5 types of stalkers were recognized: rejected, intimacy seeking, incompetent resentful, and predatory. Delusional disorders were common (30%), particularly among intimacy-seeking stalkers, while personality disorders predominated among rejected stalkers. The duration of stalking was longer for rejected and intimacy-seeking stalkers. 63% of the stalkers (N=84) made threats, and 36% (N=52) were assaultive. Threats and property damage were more frequent with resentful stalkers, but rejected and predatory stalkers committed more assaults. Committing assault was also predicted by previous convictions, substance-related disorders, and previous threats. Predatory stalkers were the smallest group (<3%), but predominantly had paraphilias and were more likely than all other groups to have previous convictions for sexual offenses and were highly likely to commit sexual assaults on the stalked victims. Ref: Excerpt adapted from Mullen PE, Purcell R, Stuart GW. Study of stalkers. American journal of psychiatry. 1999 Aug 1;156(8):1244-9.

The correct answer is: Predatory

Question **52**

Not answered

Marked out of 1.00

Homicide-suicide is a rare but serious event when an individual will kill another person, usually current or past partners, before ending their own life. Which of the following mental disorders is most associated with this event?

Select one:

- ☐ Schizophrenia
- ☐ Substance misuse
- ☐ Anxiety disorder
- ☐ Personality disorder
- ☐ Depression

Your answer is incorrect.

The most common mental health diagnosis associated with homicide-suicide in intimate or family relations, is depression. Other diagnoses are rare. Case series report that the typical homicide-suicide case involves a depressed, controlling husband who shoots his ill wife. Ref; Rosenbaum M. The role of depression in couples involved in murder-suicide and homicide. The American Journal of Psychiatry. 1990 Aug 1;147(8):1036.

The correct answer is: Depression



Question 53

Not answered

Marked out of 1.00

Autism spectrum disorders are not generally associated with a marked increase in the commission of criminal acts. Which of the following is the offence particularly associated with people with ASD?

Select one:

- ☐ Fraud
- ☐ Assault
- ☐ Theft
- ☐ Sexual offending
- ☐ Fire-setting

Your answer is incorrect.

Based on sentencing statistics, fire-setting may be the most common crime carried out by people with ASD (Dein & Woodbury-Smith, 2010). Arson is the only offence is more prevalent among all the convicted crimes committed by people with ASD compared to those without ASD. Ref: Dein, K. & Woodbury-Smith, M., 2010. Asperger syndrome and criminal behaviour. Advances in psychiatric treatment, pp. 37-43. Also see Mouridsen, S., Rich, B., Isager, T. & Nedergaard, N., 2008. Pervasive Developmental Disorders and Criminal Behaviour: A Case Control Study. International Journal of Offender Therapy and Comparative Criminology, pp. 196-205.

The correct answer is: Fire-setting

Question **54**

Not answered

Marked out of 1.00

Mr Northgate is a 48-year-old man who has been convicted for exhibitionism after exposing himself in a local park. His wife is supporting him and wishes to know what would stop him from acting in this way again. Which of the following factors is associated with him not reoffending?

Select one:

- ☐ Having exposed himself to people of different ages
- ☐ Having to appear in court
- ☐ Alcohol misuse
- ☐ Late onset of offending
- ☐ Previous convictions for theft

Your answer is incorrect.

Earlier onset of offending is associated with recidivism. Many exhibitionists have prior criminal histories and around 30% will go on to commit other sexual offences. Victimizing individuals of various ages is associated with an increased risk of reoffending. Alcohol misuse and previous convictions for other offences are associated with recidivism.

The correct answer is: Late onset of offending



Question 55

Not answered

Marked out of 1.00

Miss Olive Charleston is a 32-year-old woman with moderate intellectual disability and has been suffering from otitis externa for the past 10 days. The staff members in her care home have observed her banging her head on doors and tables. They attributed this behaviour to her intellectual disability and have been trying to reassure and distract her without success. Which of the following best describes this phenomenon?

Select one:

- ☐ Diagnostic overshadowing
- ☐ Neglect
- ☐ Retrospective diagnosis
- ☐ Institutionalisation
- ☐ Stereotyping

Your answer is incorrect.

Diagnostic overshadowing is the attribution of a person's symptoms to their mental condition, when such symptoms actually suggest a comorbid condition. The term was first used to describe the under-diagnosis of mental illness in people with intellectual disability. In recent years, the term has also been used when physical illnesses are overlooked in people with mental illness. Ref: Mason J, Scior K. 'Diagnostic overshadowing' amongst clinicians working with people with intellectual disabilities in the UK. *Journal of Applied Research in Intellectual Disabilities*. 2004 Jun;17(2):85-90.

The correct answer is: Diagnostic overshadowing

Question **56**

Not answered

Marked out of 1.00

Miss Amber O'Neill is a 48-year-old woman with a history of mild intellectual disability who usually lives in a supported accommodation. She has presented to the GP with a history of intermittent rectal bleeding over the past two weeks. She has refused a PR examination as she worries it will be painful and embarrassing although she permitted an abdominal exam. Her physical observations are normal and she appears to be clinically stable. Which of the following would be the most appropriate next step?

Select one:

- ☐ Check previous case notes to determine if she has capacity to refuse the examination
- ☐ Give her a sedative so you can perform the examination
- ☐ Ask the practice nurse to hold her while you perform the examination
- ☐ Admit to a general hospital
- ☐ Delay the examination and explain the procedure in simpler terms to the patient

Your answer is incorrect.

It would be most appropriate to try and enhance the patient's capacity by taking the additional time to explain the examination in an appropriate way and with suitable language. This may enable the patient to understand the rationale for the investigation and possibly consent. Her capacity in this situation cannot be determined from previous notes and it would not be acceptable to compel her to have the examination.

The correct answer is: Delay the examination and explain the procedure in simpler terms to the patient

Question **57**

Not answered

Marked out of 1.00

Miss Jane Williams is a 34-year-old woman with a diagnosis of Down's syndrome and moderate intellectual disability. She lives with her mother who is struggling with her physical health. Her mother is aware that Miss Williams is at an increased risk of developing Alzheimer's disease and is wondering about what she might see if this occurs. Which of the following is a more common presentation for Alzheimer's disease in people with Down's syndrome compared to those without?

Select one:

- ☐ Functional deterioration
- ☐ Disorientation to place and time
- ☐ Personality changes
- ☐ Word finding difficulty
- ☐ Memory problem

Your answer is incorrect.

Personality change manifesting in the form of behavioural and psychological symptoms appear to be a common presenting feature of Alzheimer's disease in individuals with Down's syndrome compared to the general population. This is even more frequent amongst younger individuals and those with more severe intellectual impairments. These prominent personality and behaviour changes, associated with executive dysfunction, may indicate a frontal involvement, but the pathological diagnosis is likely to be Alzheimer's in most cases. Ref: Ball SL, et al. Personality and behaviour changes mark the early stages of Alzheimer's disease in adults with Down's syndrome: findings from a prospective population-based study. International Journal of Geriatric Psychiatry. 2006 Jul;21(7):661-73.

The correct answer is: Personality changes

Question **58**

Not answered

Marked out of 1.00

Mr Mark Phillips is a 27-year-old man with a diagnosis of autism spectrum disorder and mild intellectual disability. He is currently living in a residential placement with 24-hour carers available. They have noted that he does not sleep regularly at night and this impairs his functioning during the day. They have tried to address this via non-pharmacological methods but have not been successful. Which of the following would be the most appropriate management plan for him?

Select one:

- ☐ Risperidone
- ☐ Zopiclone
- ☐ Temazepam
- ☐ Melatonin
- ☐ Quetiapine

Your answer is incorrect.

Sleep disturbance is relatively common in people with autism spectrum disorders. This is thought to be due to an abnormality in circadian rhythm and is best treated with melatonin. Ref: Andersen IM et al. Melatonin for insomnia in children with autism spectrum disorders. Journal of child neurology. 2008 May;23(5):482-5.

The correct answer is: Melatonin



Question 59

Not answered

Marked out of 1.00

Ian Moss is a 7-year-old boy who has been found to have mild intellectual impairment following an assessment carried out by the community paediatric team. He is also noted to have a long face, large ears and macro-orchidism. Which of the following syndromes does he most likely suffer from?

Select one:

- ☐ Prader-Willi Syndrome
- ☐ Klinefelter's Syndrome
- ☐ Angelman's Syndrome
- ☐ Fragile X Syndrome
- ☐ Noonan's Syndrome

Your answer is incorrect.

This description is characteristic of Fragile X. This is more severe in males and is inherited in a X-linked dominant fashion. Individuals with this disorder can also have ASD traits and ADHD symptoms, as well as difficulties with anxiety. Around 10% will experience seizures.

The correct answer is: Fragile X Syndrome



Question 60

Not answered

Marked out of 1.00

Patients with intellectual impairment can develop co-morbid psychotic illnesses. There is often a significant delay in diagnosis in these individuals. Which of the following is most likely to account for this?

Select one:

- ☐ Patients often not in contact with health or statutory services
- ☐ Difficulty in communication
- ☐ Little change can be seen in their behaviour
- ☐ Co-morbid substance use
- ☐ Patients concealing their symptoms

Your answer is incorrect.

The primary difficulty in diagnosing psychotic illness, as well as other co-morbid physical and mental illnesses, in those with intellectual impairment is communication. Individuals may not have language for their experiences or very limited ability to communicate verbally or non-verbally. They may show significant change in behaviour but interpreting this can be challenging. Ref: Welch KA, et al. Systematic Review of the Clinical Presentation of Schizophrenia in Intellectual Disability. J Psychopathol Behav Assess 2011;33:246

The correct answer is: Difficulty in communication

Question **61**

Not answered

Marked out of 1.00

Kyle Reece is a 6-year-old boy who has moderate intellectual impairment and microcephaly. He shows marked self-injurious behaviour, which has led to him requiring significant medical care. Which of the following syndromes is he most likely to have?

Select one:

- ☐ Lesch Nyhan Syndrome
- ☐ Velocardiofacial Syndrome
- ☐ Smith Magenis Syndrome
- ☐ Prader-Willi Syndrome
- ☐ Fragile X Syndrome

Your answer is incorrect.

Lesch-Nyhan syndrome or Juvenile Gout is caused by hypoxanthine-guanine phosphoribosyltransferase (HGPRT) deficiency due to X-chromosomal mutation in the HPRT gene. So it is exclusively a male disorder. Patients suffer from manifestations of high levels of uric acid production. This results in both hyperuricemia and hyperuricosuria, leading to severe gout and often, renal complications. Many affected children have moderate intellectual disability, along with abnormal involuntary muscle movements including dystonia, chorea and in some cases, ballismus. In addition to requiring wheelchairs for mobility, many will need assistance to sit. Beginning in the second year of life, a particularly striking feature of LNS is self-mutilating behaviors, characterized by lip and finger biting. It is also associated with microcephaly. Ref: <https://ghr.nlm.nih.gov/condition/lesch-nyhan-syndrome>

The correct answer is: Lesch Nyhan Syndrome

Question **62**

Not answered

Marked out of 1.00

Deletions of specific genes can lead to characteristic phenotypes. Which of the following disorders is the result of a gene deletion?

Select one:

- ☐ Lesch Nyhan Syndrome
- ☐ Tuberous Sclerosis
- ☐ Prader Willi Syndrome
- ☐ Turner Syndrome
- ☐ Fragile X Syndrome

Your answer is incorrect.

Turner Syndrome is caused by the loss of an entire X chromosome, leading to a XO genotype. Fragile X, tuberous sclerosis and Lesch Nyhan syndromes are due to gene mutations, while Prader Willi is due to uniparental disomy or genomic imprinting.

The correct answer is: Turner Syndrome



Question **63**

Not answered

Marked out of 1.00

Mrs Tanya Walters is a 75-year-old woman with an existing diagnosis of mild Alzheimer's dementia. Over the past 5 weeks her family have noticed that she has been persistent low in mood, withdrawn and has shown no interest in activities, which she used to enjoy. On assessment, she expresses feeling "useless" and constantly tired. Which of the following would be the most appropriate management plan at this point?

Select one:

- ☐ Urine dip and blood tests to exclude organic causes
- ☐ Start an SSRI antidepressant
- ☐ Review again in 2 weeks
- ☐ Arrange CBT
- ☐ Start a tricyclic antidepressant

Your answer is incorrect.

While patients with dementia can become depressed and require antidepressant medication or other interventions, they are more vulnerable than the general population to developing mood disturbance in response to organic illness, which may not be immediately apparent. It is prudent to check for organic illness before offering an antidepressant. Remember that MDD is a disorder of exclusion in the elderly. SSRIs are more appropriate than tricyclics in the older population who may have cardiac disease or be on other medications with potential drug interactions.

The correct answer is: Urine dip and blood tests to exclude organic causes



Question **64**

Not answered

Marked out of 1.00

Mr Benjamin Wyles is a 77-year-old man with a history of recurrent depressive disorder. He is currently experiencing a severe episode of depression and his appetite is very poor. His illness has not responded to 3 adequate trials of antidepressants. Which of the following is the most appropriate treatment for him at this stage?

Select one:

- ☐ Agomelatine
- ☐ Venlafaxine
- ☐ ECT
- ☐ Clomipramine
- ☐ Amitriptyline

Your answer is incorrect.

The patient is experiencing a severe treatment resistant episode of depression with marked appetite disturbance. It would be reasonable to treat him with ECT to achieve rapid improvement and address his appetite disturbance while also continuing antidepressants. Older patients can receive ECT and often respond well.

The correct answer is: ECT

Question **65**

Not answered

Marked out of 1.00

Older people who express suicidal ideation or attempt suicide are at greater future risk than younger patients. Which of the following is most associated with suicide in old age?

Select one:

- ☐ Married
- ☐ Avoidant personality disorder
- ☐ Female
- ☐ Higher openness to experience
- ☐ Histrionic personality disorder

Your answer is incorrect.

Avoidant and anankastic personality disorder are associated with increased risk of suicide in older people. Histrionic personality disorder is associated with a lower risk of suicide. All personality disorders are associated with an increased risk of suicidal ideation. Male gender, being widowed and having a lower openness to experience are associated with an increased risk of suicide in older age.

The correct answer is: Avoidant personality disorder

Question **66**

Not answered

Marked out of 1.00

Mrs Lisbeth Slander is a 79-year-old woman who has progressively developed loss of memory, visual hallucinations and intermittent irritability over the course of several months. Which of the following would be the most appropriate treatment for her condition?

Select one:

- ☐ Memantine
- ☐ Amitriptyline
- ☐ Fluoxetine
- ☐ Rivastigmine
- ☐ Diazepam

Your answer is incorrect.

This is most likely to represent Lewy Body Dementia, which is characterized by memory impairment, perceptual disturbances and mood changes. The first line of treatment is acetyl cholinesterase inhibitors, such as Rivastigmine.

The correct answer is: Rivastigmine



Question **67**

Not answered

Marked out of 1.00

Mr Yann deWael is a 70-year-old man with a diagnosis of refractory depression. He is currently experiencing a severe depressive episode, which is not responding to antidepressant treatment. Which of the following would be the most appropriate treatment for him?

Select one:

- ☐ Modafinil
- ☐ ECT
- ☐ r-TMS
- ☐ Vagus nerve stimulation
- ☐ Methylphenidate

Your answer is incorrect.

ECT has the best evidence for effectiveness in treatment resistant depression and can be used in the acute phase and as maintenance. Transcranial magnetic stimulation is increasingly being used in clinical practice. Vagus nerve stimulation is an uncommon treatment that can be used after other options have been exhausted. Stimulants have been used in treatment resistant depression but the evidence for them is limited. A meta-analysis has suggested that ECT is superior to high frequency rTMS in terms of response and remission while discontinuation was not significantly different between the two treatments. There was insufficient data on medium or long-term efficacy. Results based on 3 studies suggested that specific cognitive domains such as visual memory and verbal fluency were more impaired in patients receiving ECT than TMS. This review highlighted a lack of good quality trials comparing the long-term outcome and cognitive effects of rTMS and ECT, especially using approaches to optimize stimulus delivery and reduce clinical heterogeneity. Ref: Ren J, Li H, Palaniyappan L, et al. Repetitive transcranial magnetic stimulation versus electroconvulsive therapy for major depression: a systematic review and meta-analysis. Progress in Neuro-Psychopharmacology and Biological Psychiatry. 2014 Jun 3;51:181-9.

The correct answer is: ECT

Question 68

Not answered

Marked out of 1.00

Atypical antipsychotics are known to increase the risk of cerebrovascular complications when taken by older people. What is the factor by which atypical antipsychotics increase the risk of stroke in older people?

Select one:

- ☐ 4 times
- ☐ 1.5 times
- ☐ 5 times
- ☐ 2 times
- ☐ 10 times

Your answer is incorrect.

Older people receiving atypical antipsychotics are 2.3 times more likely to experience a stroke than those who are unexposed. This increases to 5.9 times if the individual has a diagnosis of dementia. The respective risks for typical antipsychotics are 1.7 and 3.3 times respectively. In patients with dementia, exposure to any antipsychotic increased rate of stroke by 3.5 times and by 1.4 times in people without dementia. Ref: Douglas IJ, Smeeth L. Exposure to antipsychotics and risk of stroke: self-controlled case series study. BMJ 2008; 337:a1227

The correct answer is: 2 times

Question 69

Not answered

Marked out of 1.00

Huntington's disease is a degenerative neurological condition that usually arises in middle life and leads to movement disorders, cognitive impairment, affective symptoms and sometimes features of psychosis. Which of the following is most appropriate for confirming its diagnosis?

Select one:

- ☐ CT Head
- ☐ SPECT scan
- ☐ Gene testing
- ☐ CSF assay
- ☐ MRI Head

Your answer is incorrect.

Gene testing is usually conducted to confirm the diagnosis of Huntington's. It is inherited in an autosomal dominant fashion and is due to a triple repeat expansion in chromosome 4p.

The correct answer is: Gene testing



Question **70**

Not answered

Marked out of 1.00

Mrs Abigail Baker is a 78-year-old woman with a diagnosis of Alzheimer's dementia who experiences marked insomnia, which she finds distressing during the night. She also has difficulties with recurrent falls and there are concerns that these will occur if she wanders during the night. Which of the following would be the most appropriate pharmacological intervention for her?

Select one:

- ☐ Melatonin
- ☐ Temazepam
- ☐ Zopiclone
- ☐ Zolpidem
- ☐ Quetiapine

Your answer is incorrect.

Melatonin would be most appropriate as it is not directly sedating and acts via melatonin receptors and the circadian rhythm. Any sedating agent increases the risk of falls.

The correct answer is: Melatonin



Question 71

Not answered

Marked out of 1.00

Mrs Amal Khan is a 73-year-old woman with a diagnosis of a severe depressive episode, which has not been responsive to 3 trials of antidepressant medication. Electroconvulsive therapy is now being considered and her family want to know if there are any circumstances under which this should absolutely not be used. Which of the following is a relative contraindication for ECT?

Select one:

- ☐ Diagnosis of dementia
- ☐ Parkinson's disease
- ☐ Epilepsy
- ☐ Recently diagnosed intracranial tumour
- ☐ Multiple sclerosis

Your answer is incorrect.

The sole absolute contraindication for ECT is a known pheochromocytoma. Increased intracranial pressure, current intracranial masses, recent stroke, cardiac conduction abnormalities, high risk pregnancy, aortic and cerebral aneurysms, asthma/COPD treated with theophylline are relative contraindications. Unstable angina, decompensated heart failure, severe valvular disease, uncontrolled diabetes and severe gastro-oesophageal reflux may require special consideration.

The correct answer is: Recently diagnosed intracranial tumour



Question 72

Not answered

Marked out of 1.00

With regard to supporting caregivers of patients with dementia, which of the following is the best evidence based recommendation?

Select one:

- ☐ Interventions offered from distance are not effective
- ☐ Shorter and simple interventions are not effective for most carers
- ☐ Supplying written information is the intervention most supported by current evidence
- ☐ A multicomponent approach across different times is recommended
- ☐ All carers must be referred to local Alzheimer's Society groups

Your answer is incorrect.

The evidence base for psychosocial interventions to support carers of patients with dementia is expanding, but still firm conclusions cannot be reached due to lack of well replicated, homogenous studies. According to a recent review (Jackson & Browne, 2017), "support interventions are more likely to be effective if: (1) multiple supports are offered (i.e. a multicomponent approach is taken) (2) educational interventions are designed to provide opportunity for active learning (e.g. role-play scenarios rather than 'information only') (3) the more complicated an intervention is, the less likely a carer is to adhere to it (4) distance interventions can be effective and should be considered, especially for older carers and those in isolated rural locations (5) Simply allocating a specific keyworker, supplying written only information or referring on to peer support groups are not likely to be effective". Ref: Excerpts from Jackson GA, Browne D. Supporting carers of people with dementia: what is effective?. BJPsych Advances. 2017 May;23(3):179-86.

The correct answer is: A multicomponent approach across different times is recommended

Question **73**

Not answered

Marked out of 1.00

Which of the following statements is most likely to be correct with regard to nondegenerative dementias?

Select one:

- ☐ They cannot be reversed with treatment
- ☐ Depression is not a common feature
- ☐ Multiple sclerosis is a known risk factor
- ☐ Agnosia, aphasia, and apraxia are often seen early in the course.
- ☐ Patients usually present with features of a cortical dementia.

Your answer is incorrect.

Cognitive impairment is often seen in MS and is the leading predictor of occupational disability, but unlike Alzheimer's, fronto-temporal or Lewy Body Dementia, MS related cognitive impairment is considered to be non degenerative.

The correct answer is: Multiple sclerosis is a known risk factor



Question **74**

Not answered

Marked out of 1.00

You are delivering psychotherapy to a patient who has been experiencing low mood and feelings of inadequacy. You have noticed that when you return home you tend to feel anxious and like you are not a good enough therapist. Which of the following best describes what is happening?

Select one:

- ☐ Transference
- ☐ Projective identification
- ☐ Counter-transference
- ☐ Splitting
- ☐ Reaction formation

Your answer is incorrect.

This is likely to describe projective identification with the patient projecting his or her own feelings into another individual, in this case the therapist. This is distinct from countertransference, where the patient evokes feelings in the therapist that are around the patient themselves.

The correct answer is: Projective identification



Question **75**

Not answered

Marked out of 1.00

A defence mechanism is an unconscious psychological mechanism that reduces anxiety arising from unacceptable or potentially harmful stimuli. They can be psychotic/pathological, immature, neurotic or mature. Which of the following is a neurotic defence mechanism?

Select one:

- ☐ Projection
- ☐ Humour
- ☐ Intellectualisation
- ☐ Suppression
- ☐ Denial

Your answer is incorrect.

Neurotic defenses include: displacement, dissociation, intellectualisation, reaction formation, repression, isolation, rationalisation, regression, undoing, withdrawal and social comparisons.

The correct answer is: Intellectualisation



Question **76**

Not answered

Marked out of 1.00

Family therapy is often used in situations where it is thought that interactions within the family structure are contributing to the illness that the individual is experiencing. There are various models of family therapy and similar interventions. Which of the following uses 'reframing and evaluation of roles' in the family?

Select one:

- ☐ Structural family therapy
- ☐ Couples therapy
- ☐ Strategic family therapy
- ☐ Systemic family therapy
- ☐ Interpersonal therapy

Your answer is incorrect.

Structural family therapy uses techniques such as reframing and evaluation of roles within the family. The therapist aims to "enter" into the family to understand the roles and how these are causing difficulties. They aim to disrupt and modify how family members interact and the roles they hold within the family system.

The correct answer is: Structural family therapy

Question **77**

Not answered

Marked out of 1.00

Guided self-help can be useful intervention in some conditions. It works by assisting the individual to understand their difficulties and then make positive changes, usually in keeping with an established treatment protocol. Which of the following is it most likely to be helpful for?

Select one:

- ☐ Somatisation
- ☐ Anorexia nervosa
- ☐ Dysmorphophobia
- ☐ Bulimia nervosa
- ☐ Anxiety disorders

Your answer is incorrect.

NICE recommends guided self-help in depression and anxiety disorders. It can be used in bulimia nervosa but the evidence base is not as strong.

The correct answer is: Anxiety disorders



Question 78

Not answered

Marked out of 1.00

Some psychotherapeutic interventions use hypothesising as part of their process to alleviate patient symptoms. Which of the following uses 'hypothesising'?

Select one:

- ☐ Structural family therapy
- ☐ Cognitive behavioural therapy
- ☐ Dialectical behavioural therapy
- ☐ Psychodynamic psychotherapy
- ☐ Acceptance and commitment therapy

Your answer is incorrect.

A therapist hypothesising regarding a patient's current experiences and symptoms and how these may relate to past experiences or patterns is a central element of psychodynamic psychotherapy. This term is also used often by Milan school of systemic family therapists, but in a slightly different context. In systemic family therapy, hypothesising is "the formulation of systemic 'suppositions' that guaranteed therapist activity, gave order to the interview and oriented the questioning. Hypotheses are based on positive connotation, attempted to make sense of the problem in the relational context, but did not seek to access the truth. The three systemic principles of hypothesising, circularity and neutrality are primarily employed by systemic family therapists as methods for interviewing families. These approaches also reduce the emphasis on the therapist personal qualities such as intuition, charisma and concern that can occur in a therapy setting". Ref: Excerpts from Brown JM. The Milan principles of hypothesising, circularity and neutrality in dialogical family therapy: Extinction, evolution, eviction... or emergence?. Australian and New Zealand Journal of Family Therapy. 2010 Sep;31(3):248-65.

The correct answer is: Psychodynamic psychotherapy

Question 79

Not answered

Marked out of 1.00

Mr Jeremy Blunt is a 41-year-old health executive, who is currently undergoing psychodynamic psychotherapy. During a regular session, he informs the therapist that he recently went to see a Reiki practitioner and feels that it is a "wonderful cure" and "holds the answer to all his difficulties". Which of the following would best describe this phenomenon?

Select one:

- ☐ Idealisation
- ☐ Projection
- ☐ Reaction formation
- ☐ Splitting
- ☐ Countertransference

Your answer is incorrect.

This would match best with idealization whereby an individual has an uncritically positive view of another person or object. This occurs in splitting, but with denigration, or an absolutely negative view, of another person or object. However, this is absent in this example.

The correct answer is: Idealisation



Question 80

Not answered

Marked out of 1.00

Yalom described various factors which he theorised were important to the outcome of therapy in a group setting. Which of the following describes Yalom's existential factor?

Select one:

- ☐ Taking on positive aspects of others
- ☐ Being aware of emotions of the whole group
- ☐ Understanding the futility of attempting to change the actions of others
- ☐ Understanding that one's experiences are similar to those of others
- ☐ Learning to exist as part of something larger than oneself

Your answer is incorrect.

Yalom described eleven therapeutic factors that influence change and healing in group therapy. One among them is an existential factor. It incorporates learning how to exist just as a part of something larger than oneself. This factor brings a client into the awareness that life will continue on, with pain, death, sadness, regret, and joy. By living existentially, clients learn how to accept these conditions without escaping from them. Instead, they learn how to live with them and through them.

The correct answer is: Learning to exist as part of something larger than oneself

Question **81**

Not answered

Marked out of 1.00

Family therapy is often used in situations where it is thought that interactions within the family structure are contributing to the illness that the individual is experiencing. There are various models of family therapy and similar interventions. Which of the following uses 'homework assignments' for the family?

Select one:

- ☐ Couples therapy
- ☐ Cognitive behavioural family therapy
- ☐ Systemic family therapy
- ☐ Structural family therapy
- ☐ Strategic family therapy

Your answer is incorrect.

Strategic family therapy is most associated with use of homework for families. This is often used to disrupt family patterns paradoxically, e.g. telling a child who tends to have tantrums to have them more often.

The correct answer is: Strategic family therapy

Question **82**

Not answered

Marked out of 1.00

Cognitive analytic therapy is structured into established steps, which the patient moves through over the course of the sessions. Which of the following occurs in the initial sessions of CAT?

Select one:

- ☐ Recognition
- ☐ Reformulation
- ☐ Exploration
- ☐ Exit
- ☐ Revision

Your answer is incorrect.

The initial sessions of CAT are devoted to gathering information about current difficulties and previous experiences, after which the therapist writes the patient a reformulation letter. This phase is followed by a recognition phase where the patient is helped via various means to acknowledge their difficulties and how they occur. This is followed by a revision phase where the patient and therapist plan together how the patient will cease unhelpful behaviours and patterns of thinking.

The correct answer is: Reformulation



Question 83

Not answered

Marked out of 1.00

A new Machine Learning Algorithm (MLA) is compared to the validated NAPLS-2 criteria for predicting conversion to psychosis in subjects that are identified to have a Clinical High Risk (CHR) status. 24 of 400 CHR subjects are predicted to convert in the next 2 years, according to the NAPLS-2 criteria. The MLA identifies 40 of the 400 as potential converters to psychosis, with 16 subjects being identified as converters by both the NAPLS-2 criteria and the MLA. What is the sensitivity of the MLA in predicting conversion to psychosis from the CHR state?

Select one:

- ☐ 67%
- ☐ 88%
- ☐ 56%
- ☐ 98%
- ☐ 77%

Your answer is incorrect.

To answer this question, first construct a 2X2 table. The values provided in the question can be used to fill the cells as below. Sensitivity = true positive (16) divided by total 'diseased' (24) = $16/24 = 2/3 = 67\%$

	NAPLS-2 converters	NAPLS-2 non-converters	<u>total</u>
MLA +	16	24	40
MLA -	8	352	360
<u>total</u>	24	376	400

The correct answer is: 67%

Question 84

Not answered

Marked out of 3.00

Substance misuse-signs and symptoms

Which of the following substances are most likely to be the cause of the symptoms in the below scenarios?

1. Mr Majid David is a 32-year-old man who has presented to his local A&E after having the sensation of something crawling under his skin for the last several hours. ❌
2. Miss Amber Young is a 17-year-old girl who has been brought to hospital by her friends as she has abruptly found it difficult to walk or focus her vision. On assessment medical staff note a fish-like smell. ❌
3. Mr Grayling is a 33-year-old man who has been brought to hospital by his friends in a confused and agitated state. He complained of double vision and has unsteadiness of gait. ❌

Explanation: 1) Cocaine use is associated with tactile hallucinations of insects or other creatures crawling under the skin, this is also called formication. 2) Mephedrone has a distinctive fishy smell, although it can also have odour similar to urine, vanilla or bleach. It can cause: dilated pupils, poor concentration, teeth grinding, problems focusing visually, ataxia, poor short-term memory, hallucinations, delusions and erratic behaviour. 3) Prolonged alcohol use can lead to Wernicke's encephalopathy with features of confusion, ataxia and visual disturbances.

Correct Answer is :

1. Mr Majid David is a 32-year-old man who has presented to his local A&E after having the sensation of something crawling under his skin for the last several hours. - Cocaine
2. Miss Amber Young is a 17-year-old girl who has been brought to hospital by her friends as she has abruptly found it difficult to walk or focus her vision. On assessment medical staff note a fish-like smell. - Mephedrone
3. Mr Grayling is a 33-year-old man who has been brought to hospital by his friends in a confused and agitated state. He complained of double vision and has unsteadiness of gait. - Alcohol

Question 85

Not answered

Marked out of 3.00

Patterns of genetic inheritance

Which of the following patterns of genetic inheritance are associated with the below presentations?

1. Mrs Kerry Arulpragasam is a 40-year-old woman with a long-standing history of benign masses affecting her skin and internal organs. She experiences seizures as a consequence of these masses. ✖
2. Miss Holly Wood is a 5-year-old girl with severe intellectual disability and severe self-injurious behaviour, which has required inpatient care. ✖
3. Leo Ximenez is a 7-year-old boy with mild-moderate intellectual disability. His parents are informed that his cognitive difficulties are due to the most common inherited genetic cause for intellectual impairment. ✖

Explanation: 1) This describes tuberous sclerosis, which can be caused by abnormalities in two separate tumour suppressor genes. These can occur spontaneously (more common) or are inherited in an autosomal dominant pattern. 2) This describes Lesch Nyhan syndrome, which is inherited in a X-linked recessive pattern. 3) Fragile X Syndrome is the most common inherited cause of intellectual disability and is inherited in a X-linked dominant pattern.

Correct Answer is :

1. Mrs Kerry Arulpragasam is a 40-year-old woman with a long-standing history of benign masses affecting her skin and internal organs. She experiences seizures as a consequence of these masses. - Autosomal dominant
2. Miss Holly Wood is a 5-year-old girl with severe intellectual disability and severe self-injurious behaviour, which has required inpatient care. - X-linked recessive
3. Leo Ximenez is a 7-year-old boy with mild-moderate intellectual disability. His parents are informed that his cognitive difficulties are due to the most common inherited genetic cause for intellectual impairment. - X-linked dominant

Question 86

Not answered

Marked out of 3.00

Learning disability syndromes

Which of the following syndromes are described in the vignettes below?

1. Mike Grove is a 7-year-old boy who tends to avoid making eye contact with others and has a somewhat long and thin face with large ears.

At a recent paediatric appointment he was noted to have macro-orchidism.



2. Alfie Mordant is a 6 year old boy with moderate intellectual disability who tends to be sleepy during the day but with disturbed sleep at night. His parents are very worried about his behaviour as he can cause injury to himself. At other times he hugs himself repeatedly.



3. Boris is a 14-year-old boy who was born with pulmonary valve stenosis, scoliosis, webbed neck and a sunken chest. He also has a history of excessive bruising.



Explanation: 1) Fragile X Syndrome is more common in males and is characterised by a characteristic facial appearance, ASD traits, ADHD symptoms and macro-orchidism. 2) This pattern of self-hugging, self-injurious behaviour and sleep disturbance in the presence of moderate intellectual disability is most associated with Smith Magenis Syndrome. The presence of self-hugging is a particular feature of this disorder.

Ref: <https://ghr.nlm.nih.gov/condition/smith-magenis-syndrome> 3) Noonan syndrome is one of a group of related conditions, collectively known as RASopathies caused by changes in the same cell signaling pathway (e.g. neurofibromatosis type 1, Costello syndrome). Most children diagnosed with Noonan syndrome have normal intelligence, but a few have special educational needs. A variety of bleeding disorders have been associated with Noonan syndrome. Skeletal deformities include sunken chest or protruded chest as well as scoliosis. Distinctive facial features of this syndrome include the presence of nasal philtrum (a deep groove in the area between the nose and mouth), widely spaced eyes with low-set, backward rotated ear, short chin and high arched palate. Many children webbed neck and a low hairline at the back of the neck, similar to some patients with Turner's syndrome. Ref: <https://ghr.nlm.nih.gov/condition/noonan-syndrome>

Correct Answer is :

1. Mike Grove is a 7-year-old boy who tends to avoid making eye contact with others and has a somewhat long and thin face with large ears. At a recent paediatric appointment he was noted to have macro-orchidism.- Fragile X Syndrome

2. Alfie Mordant is a 6 year old boy with moderate intellectual disability who tends to be sleepy during the day but with disturbed sleep at

night. His parents are very worried about his behaviour as he can cause injury to himself. At other times he hugs himself repeatedly - Smith Magenis Syndrome

3. Boris is a 14-year-old boy who was born with pulmonary valve stenosis, scoliosis, webbed neck and a sunken chest. He also has a history of excessive bruising. - Noonan's Syndrome



Question **87**

Not answered

Marked out of 7.00

Approaches used in psychotherapy

Therapists using different models of psychotherapeutic interventions employ distinct approaches to achieve therapeutic benefits. For each of the approach listed or described below choose the most likely type(s) of psychotherapy where it is employed, from the below list.

1. Identifying a safe place (Choose ONE) ✖
2. Outside-in approach (Choose ONE) ✖
3. Reformulation (Choose ONE) ✖
4. Rolling with resistance (Choose ONE) ✖
5. Socratic questioning (Choose TWO) ✖
- ✖
6. Task-setting stage (Choose ONE) ✖

1. Before beginning EMDR therapy, the client is often asked to identify a "safe place". This is usually a comforting memory or an image that gives a positive sense of self. This safe place is used later to help a client tolerate any upsetting sessions and bring closure to them. -
2. Most of the time we complete external actions to match our internal feelings. This is called the "inside-out" perspective. As a result, we often want to have motivation or 'drive' to precede the completion of a task; in lack there of, we watch Netflix! In behavioural activation, patients are expected to act according to a pre-made plan or written goal, so that actions precede – not follow – feeling states. This is called "outside-in" approach to behaviour.
3. In cognitive analytical therapy, following an extended assessment, the therapist and the client develop a joint description of maladaptive patterns. A diagrammatic 'reformulation' is prepared, and enactments are identified both during and outside of the sessions ('recognition'). New ways of thinking, feeling and behaving are identified as exits from the maladaptive patterns ('revision'). These 3 Rs are the core features

fo Cognitive Analytical Therapy (CAT).

4. Five principles of Motivational Interviewing include (1) Expressing empathy (2) Developing discrepancy between clients' values and their behaviors. (3) Avoiding direct confrontation. (4) Rolling with the resistance from the client instead of abrasively opposing it (5) Supporting self-efficacy and optimism.

5. The term cognitive restructuring refers to the process of challenging, and changing, irrational thoughts. Socratic questioning is one technique to encourage this. It is used in cognitive therapy as well as CBT.

6. The initial session of Strategic Family Therapy is broken down into 5 parts: the brief social stage (ice-breaking), the problem stage (family members present their main problems), the interactional stage (family members asked to talk to each other to ascertain problems in communication), goal-setting stage (family members asked to list changes they would like to see), and finally the task-setting stage (family members given a simple directive to keep involved in between sessions)

Ref: 1. Mitrani, V.B., & Perez, M.A. (2003). Structural-strategic approaches to couple and family 31 therapy. In T.L. Sexton, G.R. Weeks, & M.S. Robbins (Eds.), Handbook of family therapy (pp. 177-200). New York: Brunner-Routledge. 2. Clark GI, Egan SJ. The Socratic method in cognitive behavioural therapy: A narrative review. Cognitive Therapy and Research. 2015 Dec 1;39(6):863-79. 3. Curran, J., Ekers, D., McMillan, D., & Houghton, S. (2012). Behavioural activation. In W.Dryden (Ed.), Cognitive behaviour therapies (pp. 236-260). London, UK: Sage Publications. 4. Menon SB, Jayan C. Eye movement desensitization and reprocessing: A conceptual framework. Indian journal of psychological medicine. 2010 Jul;32(2):136.

Correct Answer is :

Identifying a safe place (Choose ONE) - Eye Movement Desensitization and Reprocessing

Outside-in approach (Choose ONE) - Behavioural Activation Therapy

Reformulation (Choose ONE) - Cognitive Analytical Therapy

Rolling with resistance (Choose ONE) - Motivational Interviewing

Socratic questioning (Choose TWO) - Cognitive Therapy, Cognitive Behavioural Therapy

Task-setting stage (Choose ONE) - Strategic Family Therapy



Question 88

Not answered

Marked out of 3.00

Long term effects of drugs of abuse

Which of the following physiological disturbances are associated with long-term use of the below drugs of abuse? You can use the options more than once.

1. Ketamine 2. Amphetamine 3. Heroin 

Explanation: 1) Ketamine has been reported as causing renal damage, in addition to its established risk of ulcerative cystitis. 2) Long-term amphetamine use can be associated with hypertension as it is a physiological stimulant. 3) Heroin and other opiates can be associated with a variety of renal complications, including membranous nephropathy, nephrotic syndrome, acute glomerulonephritis, focal and segmental glomerulosclerosis (FSGS) amyloidosis, and interstitial nephritis. Focal membranoproliferative glomerulonephritis with IgM and complement deposition is also found in heroin addicts with the nephrotic syndrome. There is a three-fold increased risk for renal dysfunction in heroin users versus non-drug users. (Ref: Singh, V. P., Singh, N., & Jaggi, A. S. (2013). A Review on Renal Toxicity Profile of Common Abusive Drugs. The Korean Journal of Physiology & Pharmacology?: Official Journal of the Korean Physiological Society and the Korean Society of Pharmacology, 17(4), 347–357)

Correct Answer is :

- 1. Ketamine - Renal membrane nephropathy
- 2. Amphetamine - Hypertension
- 3. Heroin - Renal membrane nephropathy

Question 89

Not answered

Marked out of 3.00

Data

Which of the following ways of graphically displaying data would be most appropriate for each of the following circumstances?

1. Displaying the results of several different randomised control trials which have addressed the same research question.
2. A randomised controlled trial is conducted into a new antidepressant. The authors wish to compare the rate of response across 5 age groupings.
3. A meta-analysis is conducted into the efficacy of an antipsychotic medication. The authors are aware that the various studies into its use have led to significantly different results and wish to try and gain an overview of them to check for bias.

Explanation: 1) A forest plot, also known as a blobbogram, is a graphical display of estimated results from a number of scientific studies addressing the same question, along with the overall results. 2) Bar charts are used to show comparisons among discrete categories. One axis of the chart shows the specific categories being compared, for example, age groupings, and the other axis represents a measured value, such as rates of response to medication. In comparison, a histogram only shows the distribution of continuous numerical data, for example, the number of people in each age grouping. 3) This is most likely to refer to a funnel plot. This is a graph designed to check for the existence of publication bias. They are commonly used in systematic reviews and meta-analyses. In the absence of publication bias, it assumes that studies with high precision will be plotted near the average, and studies with low precision will be spread evenly on both sides of the average, creating a roughly funnel-shaped distribution.

Correct Answer is :

1. Displaying the results of several different randomised control trials which have addressed the same research question. - Forest plot
2. A randomised controlled trial is conducted into a new antidepressant. The authors wish to compare the rate of response across 5 age groupings. - Bar chart
3. A meta-analysis is conducted into the efficacy of an antipsychotic medication. The authors are aware that the various studies into its use have led to significantly different results and wish to try and gain an overview of them to check for bias. - Funnel plot

Question 90

Not answered

Marked out of 3.00

Prevention strategies

Which form of prevention is described in each of the below scenarios?

1. Mr Lalchand is a 76 year old man from Leicester who has recently been diagnosed with Alzheimer's disease and been prescribed Donepezil. ✖

2. There is an increased nationwide drive to reduce the suicide rate, including notices at train stations and more suicide hotlines. ✖

3. Carl Hicks is a 15-year-old boy from Somerset who has been arrested for disorderly behaviour whilst intoxicated with alcohol. He is sent to an alcohol treatment programme in place of a more punitive sentence. ✖

Explanation: 1) This would be tertiary prevention; reducing the harm of symptomatic disease, such as disability or death, through rehabilitation and treatment. Primary prevention is avoiding occurrence of disease through eliminating disease agents or increasing resistance to disease. Secondary prevention is detecting and addressing an existing disease prior to the appearance of symptoms. 2) This would be universal prevention. This involves strategies that are designed to reach the entire population, without regard to individual risk factors and are intended to reach a very large audience. 3) This would be indicated prevention. These interventions identify individuals who are experiencing early signs of a disorder or problem behaviour and target them with special programmes. Selective prevention strategies target subgroups of the general population that are determined to be at risk of a condition or outcome.

Correct Answer is :

1. Mr Lalchand is a 76 year old man from Leicester who has recently been diagnosed with Alzheimer's disease and been prescribed Donepezil. - Tertiary intervention
2. There is an increased nationwide drive to reduce the suicide rate, including notices at train stations and more suicide hotlines. - Universal intervention
3. Carl Hicks is a 15-year-old boy from Somerset who has been arrested for disorderly behaviour whilst intoxicated with alcohol. He is sent to an alcohol treatment programme in place of a more punitive sentence. - Indicated prevention

Question 91

Not answered

Marked out of 2.00

Diagnosis of types of dementia

Which of the following subtypes of dementia are associated with the scenarios described below?

1. Mr Dawson is a 72-year-old man from Exeter whose family is increasingly concerned about him as he repeatedly misplaces his glasses and similar items. He has forgotten social engagements and became confused when visiting his daughter's new house.

2. Mrs Edison is 74-year-old woman from Derby with memory loss symptoms and was admitted to hospital after falling. After admission she experienced odd visual perceptions and received Haloperidol as the medical team were concerned about her having delirium. After taking this she had severe muscular stiffness and was unable to mobilise by herself.

Explanation: 1) This is most likely to represent Alzheimer's disease as he is exhibiting symptoms of progressive and disabling memory disturbance, which interferes with daily life to a significant extent. 2) This is likely to represent the neuroleptic sensitivity, falls and intermittent sensory disturbance associated with dementia with Lewy bodies.

Correct Answer is :

1. Mr Dawson is a 72-year-old man from Exeter whose family is increasingly concerned about him as he repeatedly misplaces his glasses and similar items. He has forgotten social engagements and became confused when visiting his daughter's new house. - Alzheimer's dementia
2. Mrs Edison is 74-year-old woman from Derby with memory loss symptoms and was admitted to hospital after falling. After admission she experienced odd visual perceptions and received Haloperidol as the medical team were concerned about her having delirium. After taking this she had severe muscular stiffness and was unable to mobilise by herself. - Dementia with Lewy Bodies



© 2023 SPMM Course Limited, UK.

[Refund Policy](#) | [Terms & Conditions](#)

